



YR Gaitonde Medical, Educational and Research Foundation

2006 - 2014



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The YR Gaitonde Medical, Educational and Research Foundation

The Y.R. Gaitonde Medical, Educational and Research Foundation, honours the memory of Late Shri Yeshwant Rao Gaitonde. Founded in 1985, the Foundation pioneers public health initiatives through the YR Gaitonde Centre for AIDS Research and Education (YRG CARE), and champions livelihood and food security initiatives through ECO Kitchen and Pibags.



Note from the Managing Trustee

I am pleased to present a review of our activities from April 2006 until March 2014.

In recognition that the HIV epidemic disproportionately affects women, YRGCARE devoted significant resources and effort to develop interventions and support initiatives. Our effort to include men as change agents to reduce intimate partner violence, and to engage community partners alongside, has not only been effective, but also has proven to be sustainable.

While the epidemic has slowed down in India, in general, there is a growing concern of its spread amongst men who have sex with men (MSM) and persons who inject drugs (PWID). Several new initiatives and research studies of YRGCARE specifically address their needs. During this period of review, YRGCARE's expertise has been integrated into both public sector and community partner clinics across India, to increase capacity to address voluntary testing and counseling for these most at risk populations.

Science is yet to yield a cure for AIDS or a vaccine, but now offers hope for a good quality of life to those living with HIV. Treatment is now accessible both in the public and private sector, thanks to the lowered costs and increased availability of medications. Our "Mother's

Project" provided critical treatment, peer support, and economic assistance to women affected by HIV in the five South Indian states. YRGCARE introduced free nutrition support for patients during their in patient care. A very innovative therapeutic recreational camp for children, the Camp Rainbow, now brings much needed joy and cheer to children living with HIV.

At Chennai, the YRGCARE laboratory was conferred the coveted National Accreditation Board for testing and calibration Laboratories (NABL) qualification. The laboratory also participates as a reference centre for the region for special assays and tests.

YRGCARE remains committed to testing effective prevention and treatment strategies through a range of biomedical, laboratory and community research studies. Guided by impeccable ethical standards, the research at YRGCARE continues to have huge public health impact, developing low cost assays, diagnostic tools that require significantly fewer resources, and interventions that can be taken to scale. I invite you to refer to our research portfolio in this review.

The Foundation is pleased to announce the creation of Project ECO (Enhancing Community Opportunities) during this period, promoting enterprise and livelihoods

for women in need. These projects bring opportunities to those whom we have proudly served in the last two decades: women facing partner violence, spouses of persons who inject drugs and other socially vulnerable women. The review covers these initiatives – Pibags and ECO Kitchen, projects that have brought economic autonomy to the women involved, and have served to increase their self-worth. These endeavors, along with education projects for children (Endrum Punnaigai), have all extended women and families, respect and dignity and have empowered communities.

I thank our Board of Trustees, colleagues, and consultants for their commitment and contributions. I also thank our donors, volunteers, patients, research participants and all others, for their continued support and confidence in us.

This review is a positive reflection of our many proud accomplishments. Moving forward, I remain hopeful that our efforts in prevention, care, research, advocacy and training will bring stability to the epidemic and that high-quality health care will be accessible to our communities regardless of social, sexual orientation, and gender disparities.

- Dr. Suniti Solomon

Legal Information

Status

Registered Public Charitable Trust vide registration number 190 dated 8th July 1985.

Registered u/s 12A of the Income Tax Act 1961 vide registration number 47-III dated 14th May 1990.

Donations to Trust are eligible u/s 80G of the Income Tax Act 1961 vide DIT (E) No.2 (167)/90-91 dated 2nd November 2007 with perpetual validity as provided in the 2009 Finance Act.

Registered with the Foreign Contributions Regulations Act vide registration number 075900530 dated 17th September 1990.

Administrative Office

YRGCARE

Voluntary Health Services Campus, Taramani TTTI Post, Chennai 600 113

Ph: +91 44-3910 6664. Fax: +91 44 22542939

Email: info@yrgcare.org | info@ecokitchen.org

Registered Office

15, East Street, Kilpauk Garden Colony, Kilpauk, Chennai 600010

Bankers

- > ICICI Bank Ltd., Cenotaph Road Branch
- > IDBI Bank Ltd., Alwarpet Branch
- > HDFC Bank, Cenotaph Road Branch
- > Kotak Mahindra Bank Ltd., Anna Salai Branch
- > Indian Overseas Bank, VHS Complex, Taramani
- > State Bank of India, Sastri Nagar Branch, Chennai
- > Indian Bank, Park Town, Chennai

Auditors

M/s. Venkatesh & Co.
Chartered Accountants

Internal Auditors

M/s. DPV Associates
Chartered Accountants

FCRA Auditors

M/s. Kandan & Nambi
Chartered Accountants

Board of Trustees

Name	Occupation	Since-Till
Dr. Ms. Suniti Solomon Managing Trustee	Medical Microbiologist	1985 till date
Prof. SP Thyagarajan Trustee	Academic Administrator	2006 till date
Dr. M.K.Mani Trustee	Consultant Nephrologist	2007 till date
Dr. Sunil Suhas Solomon Trustee	Epidemiologist	2008 till date

Former Trustees	Occupation	Since-Till
Late Mr RY Gaitonde Founder Trustee	Industrialist	1985-2000
Late Mr DY Gaitonde Founder Trustee	Management Advisor	1985-2007
Late Mr KY Gaitonde Founder Trustee	Industrialist	1985-2005
Late Mr VY Gaitonde Founder Trustee	Senior Technocrat	1985-2013

The Trustees, Staff and others connected with the Foundation thank the Founder Trustees for their vision, and the invaluable guidance and support provided by them.

Institutional Review Board

The Institutional Review Board (IRB) (IRB 00001423/FWA 00000672) governs YRGCARE' faculty and staff in the discharge of its responsibilities for protecting the rights and welfare of human participants in research and evaluation assessments conducted at or sponsored by YRGCARE. The IRB has the authority to review and approve, comment, and/or disapprove any research/evaluation activity occurring at YRGCARE. This is done through a careful review of protocols, informed consent documents, case report forms, other research/evaluation documents and ethical considerations and practices based on national and international ethical guidelines. During 2006-14, 108 meetings were held and 220 new or ongoing studies were reviewed.

S.No.	Name	Qualifications	Affiliation
1	Professor Dr S A Jayakar Paul Chairperson	MBBS, MD	Retd. Director, Dept. of Internal Medicine, Madras Medical College
2	Dr. V. Kumaraswami Co-Chairperson	MBBS, MD, MNAMS, PhD	Former Director-in-Charge, National Institute for Research in Tuberculosis & National Institute of Epidemiology, Indian Council of Medical Research, Chennai
3	Mr. Arvind Subramaniam	B.L	Advocate, Madras High Court
4	Mr. K. Arumugam	B. A.	Lay person – Community
5	Mr. Benjamin Franklin	M.S.W	Social Worker
6	Dr. Jayapaul Azariah	Ph.D	Retd. Prof. Of Zoology-University of Madras
7	Dr. Jyotirmay Biswas	M.D	Ocular Pathologist, Sankara Nethralaya, Chennai
8	Dr. Malathy Doraisamy	Ph.D	Professor of Economics, Indian Institute of Technology
9	Dr. P. Kuganantham	MBBS, DPH, WHO Fellow, MPH (Johns Hopkins, USA), DTM&H (LSTM&H.UK), FISC	City Health Officer (Retd.), Health Department, Corporation of Chennai
10	Dr. A. Nalini	MBBS, MD, Masters in Bioethics	Director of Education, Saveetha University, Saveetha Nagar, Thandalam,
11	Dr. T. Rajkumar	MD, DM, PhD	Director of Research, Cancer Institute, Chennai
12	Dr. R. Ramakrishnan	Ph.D	Deputy Director National Institute of Epidemiology
13	Dr. Saradha Suresh	MD, Ph.D, FRCP (Glas),	Retd. Director & Superintendent, Institute of Child Health & Hospital for Children, Chennai
14	Ms. Sarah Chanda	M.A.	Social Worker
15	Ms. Sheila Jayaprakash	B. L.	Advocate, Madras High Court
16	Dr. S. Shantha	MBBS, MD, Ph.D.	Retd. Director, Institute of Microbiology, Madras Medical College
17	Dr. M. Suresh Kumar	M. D., MPH	Consultant Psychiatrist
18	Dr. S. Swarnalakshmi	M. A., M.Phil, P.G.D.P.M., P.G.D.B.M., P.G.Diploma in Bioethics, Ph. D.	IRB Manager, YRGCARE & Member Secretary, YRGCARE IRB

Community Advisory Board

Community Advisory Board (CAB) consists of representatives of the general public who foster partnerships between the general public and the clinical trial researchers who conduct clinical trials.

The members are both direct and indirect stakeholders. Direct stakeholders include those who are impacted by the health issue that is researched. Indirect stakeholders are those individuals involved in a variety of occupations with a genuine interest in community growth, health issues, research ethics: these include doctors, lawyers, and exemplary citizens with a demonstrable social conscience. The members voice their concerns based upon their involvement in their respective communities and provide feedback on proposed research questions and study design.

S.No.	Name	Affiliation
1	Fr. Xavier Alphonse SJ CAB Co-Chair	Academician, Catholic Priest and NGO Director
2	Mrs. Malini Balakrishnan CAB Co-Chair	Hospital Administrator
3	Dr. Shakuntala Suresh Kumar	Consultant Psychiatrist, Chennai
4	Ms. Anitha Sumanth	Advocate, Madras High Court
5	Ms. Alamelu	Community – Peer Educator
6	Ms. Amulu	Community – Peer Educator
7	Ms. Dhanalakshmi	Community – Peer Educator
8	Ms. Jeeva	Community – Peer Educator
9	Ms. Mala	Community – Peer Educator
10	Ms. Pushpa	Community – Peer Educator
11	Mr. Ramesh	Community – Peer Educator Affiliate: YRGCARE Clinic, Royapuram
12	Ms. Sasikala	Community – Peer Educator
13	Mr. Sasi Kumar	Community – Peer Educator
14	Mr. Shokenraj	Community – Peer Educator
15	Ms. Usha	Community – Peer Educator

Human Resources

The Y.R.Gaitonde Medical Educational and Research Foundation has an able human resource team that is highly committed, capable and qualified to respond to the HIV pandemic. Interns and volunteers, consultants and collaborators from India and abroad, complement core staff.

The Foundation continually invests in training and capacity building of its staff members.

The Foundation has policies and procedures on prevention of discrimination and sexual harassment, code of conduct and conflict of interest. It encourages diversity amongst its staff and offers employment opportunities for persons living with HIV and those with disabilities.

Grade	Female	Male	Total
Director, Senior Managers and Advisors	1	7	8
Managers	5	16	21
Senior Associates	12	21	33
Associates	34	32	66
Assistants	86	54	140
Contract Staff	60	22	82
Total	198	152	350

Gender Diversity as at March 31, 2014



YRGaitonde Centre for AIDS Research and Education

Vision

That people with HIV and AIDS, and their families live with dignity, and there are no new infections.

Mission

To respond to prevention, care and research needs of the country.

YRGCARE's expertise includes HIV clinical care for over 19000 patients living with HIV, outreach prevention services, a NABL accredited laboratory, medical and counseling training, and clinical, laboratory and behavioural research. Its wide array of services include HIV treatment, care and support; sexuality and treatment education; anonymous testing for HIV/STI; matrimonial services; reproductive health and obstetric care; pediatric care and therapeutic recreation camps for children with HIV; and clinics for men having sex with men and people who inject drugs.

Awards, Honors and Recognitions

Awards & Honours

2008	Mr. AK Srikrishnan	Omololu Falobi Award for Excellence in HIV Prevention Research Community Advocacy
2009	Dr Suniti Solomon	Sivananda Eminent Citizen Award (Significant contribution to HIV/AIDS prevention), Sanathana Dharma Charitable Trust, Secunderabad
2009	Dr Sunil Suhas Solomon	Charlotte Silverman Fund Award, Johns Hopkins University, Baltimore, USA
2009	Dr Sunil Suhas Solomon	Delta Omega Honour, Johns Hopkins University, Baltimore, USA
2009	Dr Sunil Suhas Solomon	Phi Beta Kappa Award, Johns Hopkins University, Baltimore, USA
2012	Dr R. Vignesh	Young Investigator Award –CROI 2012, Seattle, USA.
2012	Dr. Suniti Solomon	Lifetime Achievement Award (The TN Dr.MGR Medical Univ)
2012	Dr. Suniti Solomon	Lifetime Achievement Award by Indian Association of Medical Microbiology, Pondicherry Chapter
		“National Women Bioscientist Award”, Department of Biotechnology (Senior Category) for the year 2009 in recognition of outstanding contributions in the field of HIV/AIDS – Biomedical research.

AIDS Clinical Trials Group Award (2014)

YR Gaitonde Centre for AIDS Research and Education (YRGCARE) was ranked No. 2 among all the International clinical research sites of AIDS Clinical Trials Group (ACTG) / National Institute of Health (NIH) for scientific contribution and enrolment for the period 2007-2013.

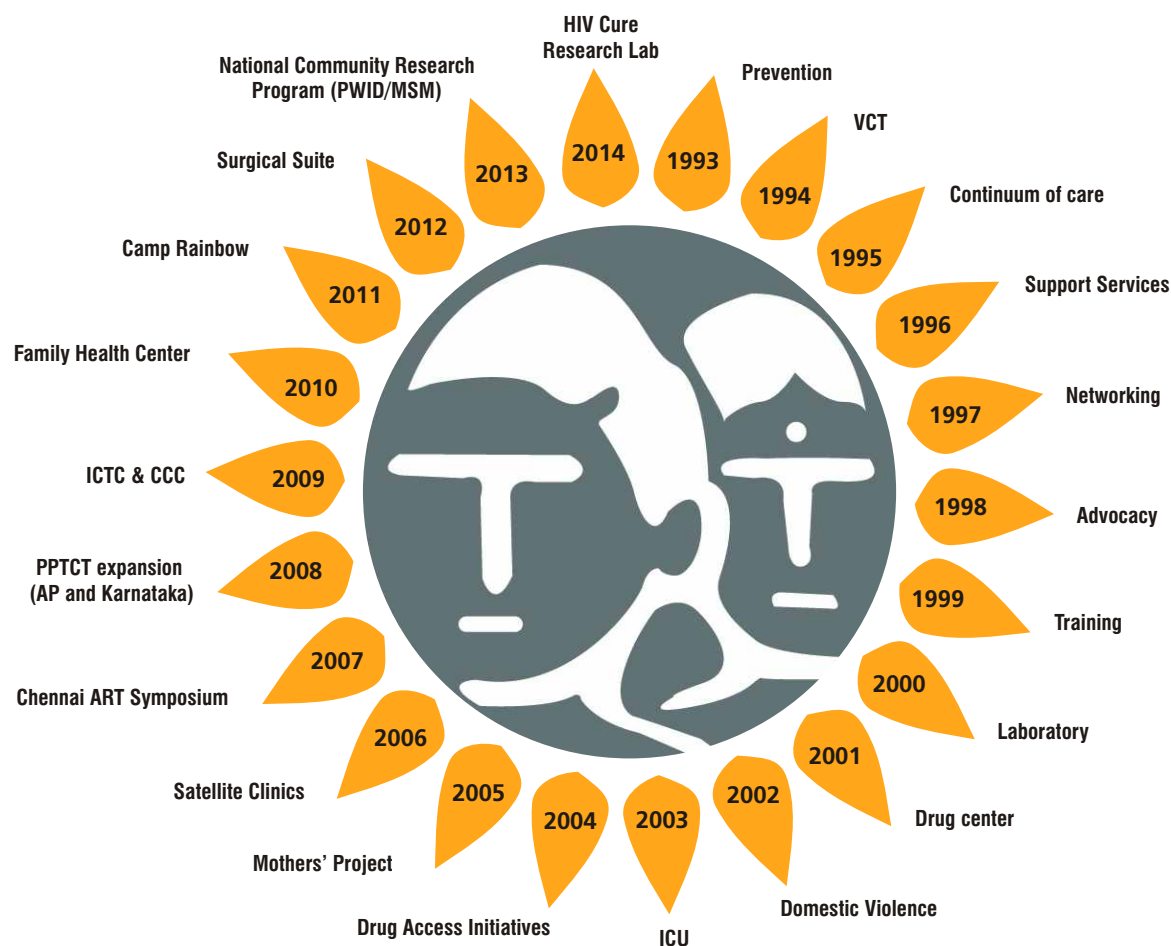
25 years of
HIV / AIDS
in India



Commemorating A Journey Through 25 Years

YRGCARE commemorated 25 years of HIV in India on January 21, 2012 at Chennai, acknowledging the progress in the response to HIV/AIDS since Dr Suniti Solomon and her team first documented evidence of HIV in India in 1986. The meeting discussed the persisting challenges ahead. Several national and international experts participated and shared their views and insights

YRGaitonde Centre for AIDS Research and Education (YRG CARE)

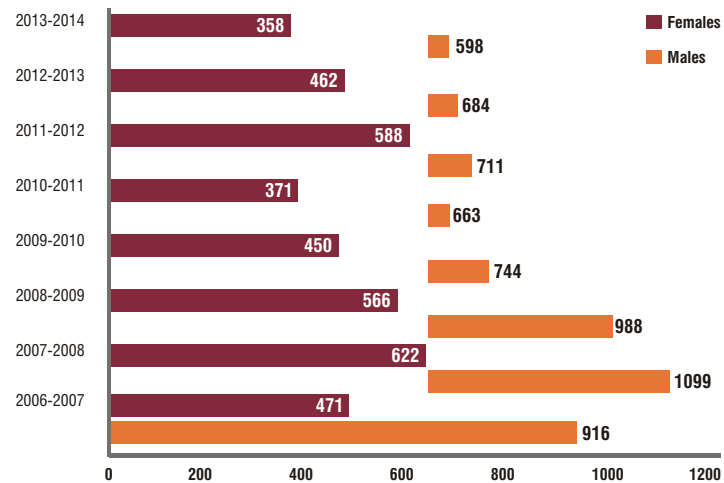


Hospital

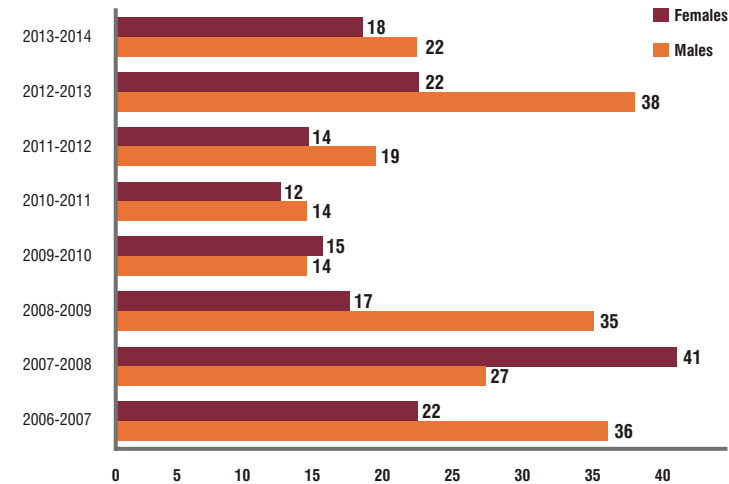
Diagnosis, prevention, treatment and support services for HIV and opportunistic infections and setting guidelines for treatment options and management of HIV in resource limited settings.



YRGCARE has been providing affordable, client-centered services since 1996. These services not only meet the changing needs of people living with HIV, but also address the prevention and support needs of spouses, partners, families and others affected by the friends. Between 2006-2014, **10291 adults and 366 children** registered for services.



YRGCARE Registrations (2006 – 2014) Adults

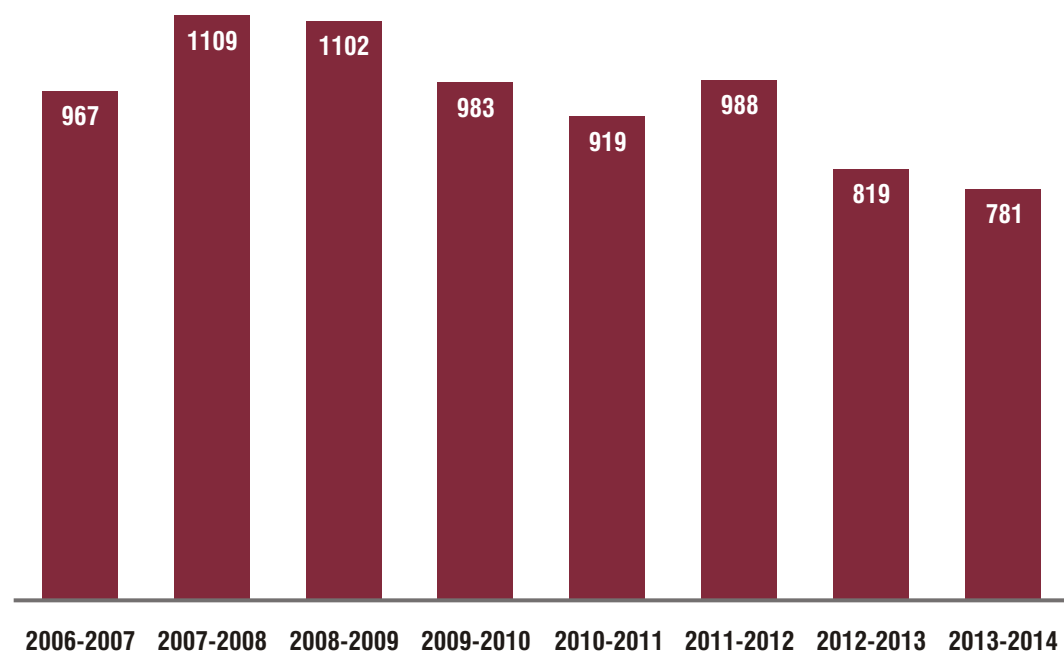


YRGCARE Registrations (2006 – 2014) Children

Increased Access to Care

YRGCARE expanded its clinical care and support programmes both at Chennai and its satellite sites in Southern India. Free anonymous HIV testing, medical support for opportunistic infections and viral load testing was made possible for men, women and children through a gift of the Annenberg Foundation (2008-09).

Trend of ART Initiation (2006 – 2014)



Key Highlights (2013)

Global Impact:
Dr. N Kumarasamy, Chief Medical Officer and Senior Researcher, YRGCARE contributed to the development and publication "Consolidated guidelines on the use of antiretroviral drugs for treating and preventing HIV infection, Recommendations for a public health approach, June 2013, World Health Organisation."

Read more:

<http://www.who.int/hiv/pub/guidelines/arv2013/download/en/>

Expanding the eligibility criteria for ART and the options for using ARV drugs creates opportunities to save lives and reduce HIV transmission.

Health and Nutritional Support (2004-2006)

The Andrew Zeigler Foundation, San Francisco, created a calendar enlisting “365 Good Friends” whose financial gift for each calendar day of the year provided YRGCARE inpatients with nutritious food and supported families with support for medication for common infections, blood tests and medical imaging such as scans. The Foundation also bestowed an endowment subsequently to carry out the nutrition project on an ongoing basis.

Graduated Cost Recovery (GCR) Model (2005-2010)

Supported by Global Fund for AIDS, TB and Malaria, this unique model of access to care was comprehensive, covering drugs, diagnostics, patient education and adherence support. A total of 3010 patients were served over a five year period. The GCR model was not a full risk-pooling mechanism, especially since the clients were not required to pay for the services in advance. Based on an objective assessment of ability to pay based on household income, and an empathetic “willingness to pay” query, clients were allocated to one of four tiers – 100%, 75%, 50% and zero co payment for drugs with all other services being offered free. Across all tiers, patients showed demonstrable gains in CD4 counts, a commonly used surrogate marker for health outcomes.

Voluntary Counseling and Testing Centre, Namakkal (2006-09)

YRGCARE operated this facility in conjunction with the GFATM project to spread awareness about HIV in the communities in Namakkal and encouraged walk-in for voluntary testing. In addition to outreach staff and a nurse, this centre also featured specialty clinics offered by YRGCARE medical team on a regular basis. YRGCARE linked with community partner organizations and networks of people living with HIV to provide expanded HIV services. Over 2000 individuals were served at this centre over three years.

Nellore Primary Care Centre (2006 till date)

The Nellore Primary Care Centre, was established by YRGCARE in 2006 in collaboration with the Indian Red Cross Society (IRCS) offering a range of services including day care, pharmacy, individual and couple counseling, community mobilization and support to other NGOs. This centre was created to reduce the burden of time and cost on our patients from Andhra Pradesh.

HIV Care partner scale up (2006-2012)

With financial support from American India Foundation, USA, YRGCARE facilitated capacity building in Desire Society, Medhak and CHUM, Miriyalgunda to strengthen their response to the HIV epidemic, offering voluntary counseling and testing services, medical care and psychosocial support for women living with HIV.

Well Women Clinic (2006-09)

With financial support from the MAC AIDS Fund and technical support from the AIDS Project Los Angeles, USA, the Well Women's Clinic at YRGCARE helped to stabilize over a hundred economically challenged families affected by HIV disease by focusing on the needs of women living with HIV, or were partners of men living with HIV, or were mothers and grandmothers of a child infected. The project empowered women through emotional support groups and provided medical, nutritional and financial aid for these women. By taking care of themselves first, women became powerful forces within their own communities for education, prevention, testing, care and stigma reduction.

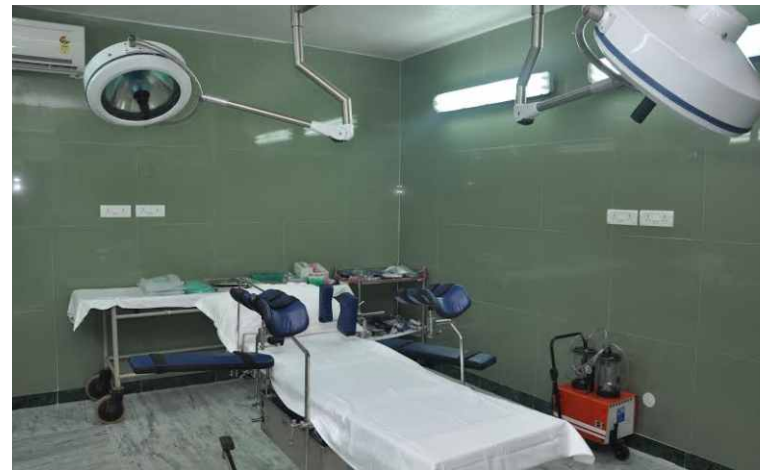
Prevention of Parent-to-Child Transmission of HIV (Since 2006)

4.3% of HIV infections occurred in new borns in India and for those under the age of 15, mother to child transmission remains the most significant route of infection (NACO, 2010). An estimated 56,000 babies are born with HIV every year in India (NACO 2012).

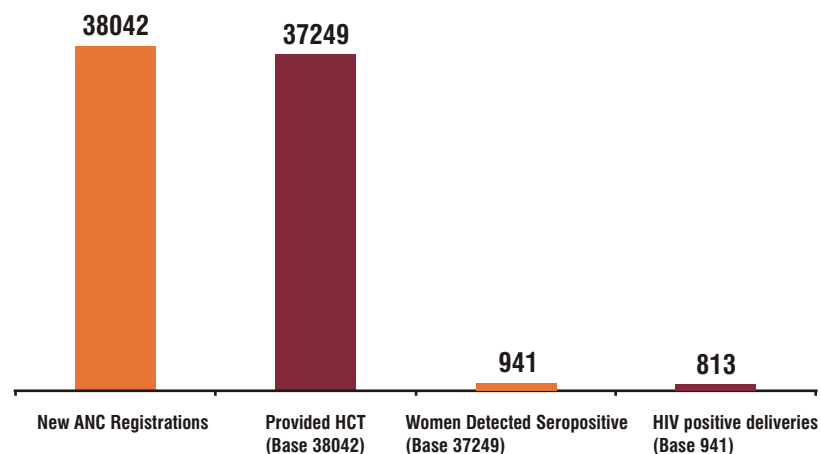
While prevention strategies are available anytime during pregnancy, at delivery and post partum, the lack of adequate PMTCT services in the private sector – where over 50% of institutional deliveries occur- and continuing substantial home deliveries (approx. 60%) contribute to low access to these strategies. Project CONNECT (2006-2011), a public-private partnership funded by USAID, was a coalition consisting of Population Services of India (PSI), YRGCARE, Federation of Indian Chambers of Commerce and Industry (FICCI) and the Karnataka Health Promotion Trust (KHPT). Drawing on the experience on PPTCT that spanned a decade, YRGCARE/Project Connect established three models of PPTCT in the private sector at Chennai, Vishakapatnam and Bangalore, to challenge discriminatory practices, understand cost of care and devise interventions to reduce stigma. The key components of this project were demand creation, expanding ARV prophylaxis, case management for mother baby pair, optional caesarean section for the mother and nutrition support for the infants. YRGCARE laboratory also supported the early diagnosis of new born with DNA PCR.

Surgical Suite

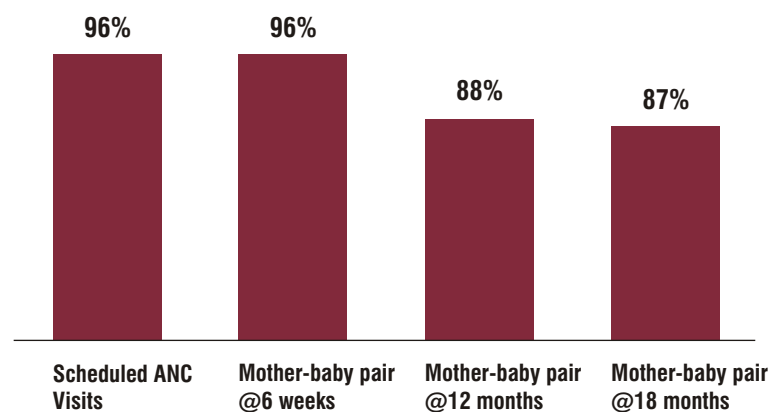
A surgical suite at YRGCARE for prevention of mother to child transmission of HIV was gifted by Sun Foundation. Obstetricians received 'hands on training' on safe delivery practices through a grant from the MAC AIDS Fund. Women living with HIV have opportunities for safe and comfortable antenatal, obstetric and postnatal care.



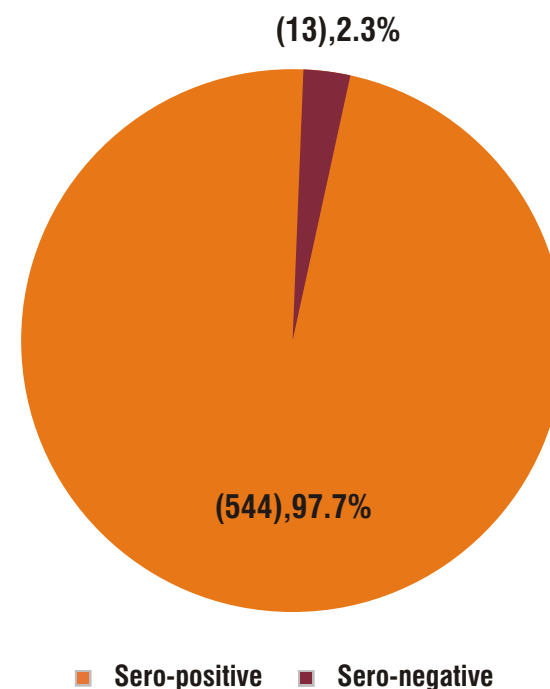
Programme Outcomes Private PMTCT Facilities 2007 - 2011



Follow up Rates at Connect PPTCT Models as a Result of Case Management Approach



HIV status of infants born to HIV Positive mothers at PPP facilities, N 557, June 2011

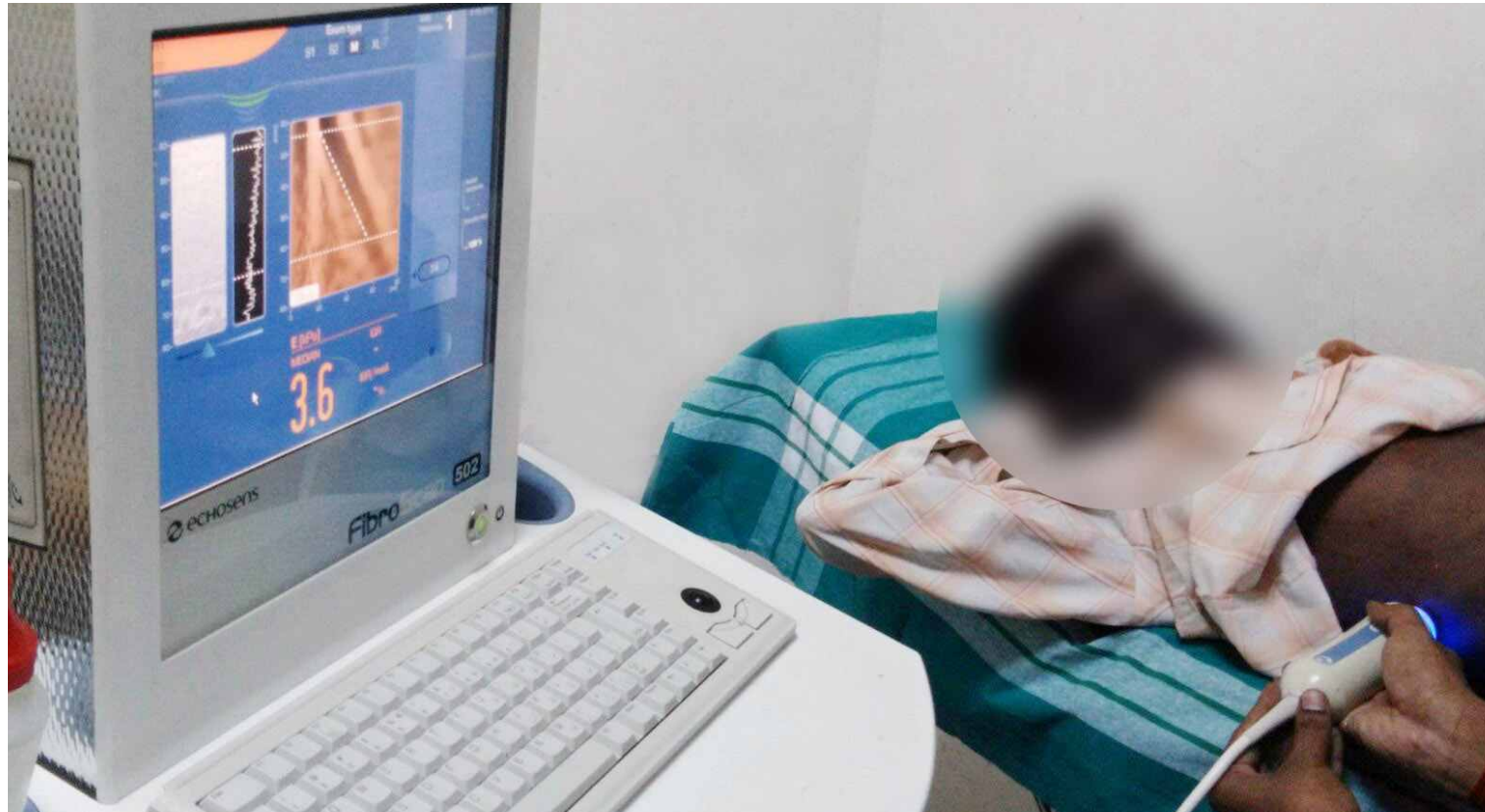


Royapuram Clinic, North Chennai. (2007 onwards)



The Royapuram Clinic, North Chennai, brings healthcare services to the general population, transgender persons, Men who have sex with men (MSM) and people who inject drug (PWID) all under one roof. The facility challenges myths and breaks barriers that foster stigma related to HIV care and sexual health clinics. The facility was inaugurated by Dr. Richard Krause, NIH in July 2007. As of March 2010, over 1500 PWID have received care at this centre. The Royapuram Clinic provides the neighbouring community with several health care services.

In 2012, YRGCARE dedicated a Fibroscan, to scan the liver to detect the extent of liver damage- fatty liver, fibrosis or cirrhosis. PWID receive a free scan every 3 or 6 months depending upon whether they are living with HIV and receive antiretroviral therapy.



Over 400 PWID have received about 3300 Fibroscan tests in over 2 years; (2012-14)

Community Care Centre, Royapuram, North Chennai (2008-2011)

Under public-private partnership program with the National AIDS Control Program (NACP-III), YRGCARE together with the Tamil Nadu State AIDS Control Society (TANSACS), initiated a short-term Community Care Center for people living with HIV. This facility offered healthcare, dietary assistance, shelter, psychosocial support and counseling, and re-entry of men and women living with HIV back to their families that previously lacked knowledge on HIV and were not accepting. This centre was awarded 'A Grade CCC' by NACO (2010).



Family Health Centre (2010 onwards)

On March 11, 2010, YRGCARE dedicated The Family Health Centre, at the VHS Campus, adding approximately 12,000 sq. ft. to the existing medical centre. The centre was a gift from the MAC AIDS FUND. It houses a surgical suite, emergency care facility, patient waiting hall, pharmacy, outpatient and in-patient services, clinical laboratory and specialty consultation for vision care and oral health.

Camp Rainbow

Residential and day camps for children living with HIV

Goal is to make camping magical for these children and help them learn essential life skills

The Therapeutic Recreational model uses child centric approach and child behaviour management techniques



Since 1999, YRGCARE has been offering special clinics for children living with HIV. The purpose of these clinics is to give these children a platform to learn more about HIV and to engage with their caregivers. The clinics use arts and crafts, puppetry and other games. It was during these clinic sessions that YRGCARE appreciated the need for a more focused and specialized program that can make adopting knowledge and practice easier for children living with HIV.

This Camp Rainbow took form as a residential and day camp program that brings children with HIV to an outdoor location and uses a therapeutic recreational model to teach them essential life skills and help them make friends.

These camps help children living with HIV reach beyond illness, to discover joy, confidence and a chance to recover their childhood. It restores their hope and renews their sense of possibility. Through Camp Rainbow, children learn to not only better manage their illness, but also able to thrive socially and emotionally on their way to becoming healthy, self-confident adults.



A typical 5-day residential camp connects the children to their trainers who work with them on

- > Traditional camp activities that promote peer support, self confidence and resilience.
- > Life skills training that enhance campers' understanding of HIV and their own health status.
- > Lifestyle changes that encourage hygiene, nutrition, and well being.
- > Medications and adherence to treatment thereby reducing stigma.

YRGCARE is happy to say that we have seen these camps rekindle the children's spirit of joy and curiosity and enhance their self-esteem and social connectedness. Campers have fulfilling experiences, are eager to participate and meet with new friends. Camp Rainbow rebuilds confidence and promotes individuality through laughter, exploration and friendship. Campers are nurtured by their peers and staff and are encouraged to challenge themselves to reach for the best.



Camp Rainbow transforms children from patients to fun-loving, capable, thriving kids!

Some of the immediate results from these camps include

- > 55% improved adherence
- > Lowest quartile had the most increase in HIV knowledge upto 144%
- > 80-90% increase in the attitudes towards HIV and taking ART medications
- > 39% improved health & hygiene
- > Coping with HIV improved by 33%
- > Social connectivity showed a 3-fold increase
- > Peer Relationship increased by nine-fold



Training and Mentorship

Volunteers drawn from colleges, healthcare institutions and NGOs serving children with HIV receive instructions in child-centric techniques that provide children living with serious illnesses a fun filled experience through interactive, structured intentional programming.

Sustaining Camp Rainbow

Camp Rainbow participation comes at zero cost to these children. All costs relating to children's travel, boarding, lodging, meals, training material and onsite medical assistance are included. In addition to this, the project also bears the cost of recruitment of campers, medical screening, preparation of training course and cost of trainers.

YRGCARE is able to achieve this goal through generous donations from its patrons and hopes to bring more and more children to camp through continued support from its patrons and their associations.

"BECAUSE I AM HAPPY"

Naina, travelled a long way to reach camp. When she got off the bus, she was beaming and a source of smiles and joy for everyone. Little Naina wanted to be happy, she was longing for a friend and to know more about HIV. Full of giggles and fun, she told her new friend, "Because I am happy!"

It is difficult to have a dry face at the end of camp. Each child and each adult has a story to tell and an adventure to share.

The India Partnership Program

In a strategic partnership, SeriousFun Children's Network and YRGCARE, through its India Partnership Program (IPI), work with other key organizations in India to build capacities and support camp programming thereby creating opportunities for children with HIV in other parts of India to experience camp.

Local partners include Community Health Education Services (CHES), Chennai (since 2011) and Asha Kirana, Mysore (2012). A new camp partnership is scheduled for summer of 2014 with ASHA Foundation, Bangalore (2014).

Between 2011-13, the India Partnership Initiative has served more than 364 campers through 5-day residential camps, conducted capacity building sessions for over 100 adults and organized regular follow up programs within the districts throughout the year. Currently, the focus is on serving children with HIV and AIDS. Each year, the number of children served increases and our goal is to serve 300 children during summer in 2014-15.

SeriousFun Children's Network's Global Partnership Program



Camp Rainbow is part of SeriousFun Children's Network Global Partnership Program started in 2008 which covers 13 countries in Africa, Asia, South America and the Caribbean.

In 1988, the legendary actor Paul Newman, founder of SeriousFun Children's Network (SeriousFun), had an idea: Imagine if kids with serious illnesses had the chance to, as he used to like to say, "kick back and raise a little hell." A camp was born in the United States and then a global movement - the Global Partnership Program (GPP) – that connects 13 countries in Africa, Asia, South America and the Caribbean, serving children with serious illnesses where medical and psychosocial needs often outstrip the available local resources.

SeriousFun helps create opportunities for children and their families to reach beyond serious illness and discover joy, confidence and a new world of possibilities, always free of charge.

Infectious Diseases Laboratory

Quality assured and affordable diagnostic laboratory with national and international recognition.

The YRGCARE Infectious Diseases Laboratory is a state-of-the-art laboratory that offers high quality and affordable services to patients. Its staff bring diverse expertise to the laboratory to efficiently cater to the needs of HIV patients. Apart from its support to the clinical services in HIV disease management, the other important laboratory activities are:

- > Engaging in basic scientific research in the various aspects of HIV and other infectious diseases such as viral hepatitis and TB.
- > Providing educational training to young scientists leading to PhD degree in Medical Microbiology (affiliated to University of Madras).
- > Conducting various academic/training program such as Good Clinical Laboratory Practices (GCLP) workshop on International Standard (ISO 15189: 2012), International Science Symposium on HIV and Infectious Diseases (ISSHID) and Certificate course on HIV Testing and Counseling (HTC).

The National Accreditation Board for Testing and Calibration Laboratories (NABL) is a biannual certification awarded by the Department of Science and Technology, Government of India. YRGCARE Infectious Diseases Laboratory has been assessed and accredited in accordance with the standard of ISO 15189 for medical laboratories-particular requirements for quality and competence in the field of medical testing since 2007.

Since 2009, recognized as a Scientific and Industrial Research Organisation (SIRO) by the Department of Scientific and Industrial Research (DSIR), Government of India, New Delhi. Supports several clinical and prevention trials funded by the US National Institutes of Health (NIH), including HPTN and ACTG networks. The laboratory is qualified for various certifications that enables its participation in these networks.



Polychromatic flowcytometry facility with 8-color parameters & Clinical Biochemistry Laboratory.



Automated PCR processing system (ABBOTT sp2000)
Equipment - ABBOTT sp2000



Molecularbiology and Genotyping Laboratory

International Recognition

Since 2006, this laboratory serves as a International Regional HIV Genotyping Specialty Laboratory (performing HIV drug resistance assay) and is one of 5 international laboratories approved by the NIH Division of AIDS.

Additionally, in 2012, the Laboratory was recognized as a Regional TB Diagnostic Laboratory (RTBDL) and is one of six of the 14 competing global sites. As a RTBDL, the laboratory performs protocol-specific TB diagnostic assays and serves as a regional laboratory for providing support to the scientific TB agenda and TB therapeutic agenda of other US NIH-funded Adult Clinical Trial Group (ACTG), The International Maternal Pediatric Adolescent AIDS Clinical Trials (IMPAACT) and HIV Vaccine Treatment Network(HVTN).

Read more: <https://actgnetwork.org/Laboratories>

No one observing or working on HIV need to be reminded of the impact advances in research pursuing a cure would have on the global epidemic. To this end, the “YRGCARE HIV Cure Research Laboratory” was inaugurated on January 30, 2014 by Professor Françoise Barré-Sinoussi, President of International AIDS Society, Geneva and Nobel Laureate credited with the co-discovery of HIV. The main aim of this laboratory is to measure and characterize HIV-1 viral reservoirs that cause rebounding of viral level.



Accomplishments

Doctoral Program in Medical Microbiology affiliated to the University of Madras

Year	Scholar	Specialisation	Research Title
2008	Dr. S. Uma	Medical Microbiology	Molecular adaptive evolution in HIV-1 subtype C reverse transcriptase and protease regions among South Indian patients.
2008	Dr. M. Sundaram	Bio-Chemistry Microbiology (inter disciplinary)	Bio-chemical changes among persons with HIV disease in Southern India.
2008	Dr. M. Vidya	Microbiology	Genotypic HIV-1 drug resistance among patients failing anti-retroviral therapy in South India.
2009	Dr. H. Syed Iqbal	Medical Microbiology	Drug resistance, polymorphisms and phylogenetic analysis of HIV-1 pol gene from injecting drug users and VCT clients in Chennai.
2012	Dr. R. Vignesh	Medical Microbiology	Mycobacterial and immunological studies on HIV/TB co-infection and immune reconstitution inflammatory syndrome.

Currently 9 students are pursuing their PhD degree; 3 have received direct fellowship from DST (INSPIRE program), CSIR and UGC.

Year	Name	Award
2011	Dr R. Vignesh	International Scholarship Award, CROI 2011, Boston, USA; poster presentation
2012	Dr S. Saravanan	Scholarship Award to present paper at XIX International AIDS Conference in Washington DC, USA, AIDS International Training and Research Program (AITRP) – Fogarty, Brown University, RI, USA.
2013	Prof. Suniti Solomon	IAMM best research paper award 2014 for Solomon S. et al. "A Rapid and Low-cost Microscopic Observation Drug Susceptibility (MODS) Assay for detection of TB and MDR-TB among HIV infected individuals in Southern India". Indian Journal of Medical Microbiology 2013; 31(2):20-27.
2013	Dr S. Saravanan	International Scholarship Award, IAS 2013 HIV Cure Symposium & 7th IAS Conference on HIV Pathogenesis, Treatment and Prevention (IAS 2013), Kuala Lumpur, Malaysia 29 June to 3 July 2013.
2013	Dr H. Syed Iqbal	Travel scholarship award, 11th ICAAP, Bangkok, Thailand
2013	Dr R. Vignesh	Travel scholarship award, 11th ICAAP, Bangkok, Thailand
2013	Dr R. Vignesh	Travel scholarship award, NIH-Workshop on Scientific Paper Writing, Sanjay Gandhi PG Institute of Medical Sciences (SGPGI), Lucknow
2013	Dr S.Saravanan	International Delegate Scholarship Award, CROI 2014, Boston, MA

Other Accomplishments

- Member of the Initiative for Promoting Affordable and Quality TB Tests (IPAQT), a joint venture facilitated by Clinton Health Access Initiative (CHAI);
- Regular re-accreditation by the National Accreditation Board for Testing and Calibration Laboratories (NABL), Government of India, New Delhi for the ISO15189 since 2008; the current certificate is valid until November 2016.
- Renewed seven-year-grant for Regional TB Diagnostic Laboratory of the ACTG/ IMPAACT/ HVTN/NIH.
- Empanelled for the diagnostic services for Southern Railway Hospitals, Airport Authority of India and Jet Airways.
- Dr. P. Balakrishnan, Chief Lab. Manager, contributed to the revision of laboratory diagnostic guidelines of the World Health Organization, New Delhi.
- Lab endorsed by CAVD/CA-VIMC to perform HIV Neutralizing antibody assays (2011).

Equipment Installed

- Biochemistry Autoanalyser AU 480 (Beckman Coulter).
- PROFLEX Multi-gradient PCR Thermocycler
- Refrigerated Centrifuge – Thermo Scientific Sorvali ST 16R
- Ultra Deep Freezer -70°C (n=3)
- ABI 7500 Fast block Real-Time PCR (Murasoli Maran Memorial Foundation; August 2012)
- New Bio-specimen Repository Facility (January 2013) - 16 deep freezers (- 70+ 5 °C); Dr. Ramesh Paranjape, Director, NARI, ICMR, Pune.
- Veriti PCR in Molecular biology (July 2013).
- FC500 5 color Flowcytometer (Beckman Coulter Inc., USA)
- Cepheid GeneXpert RealTime PCR system. Ice flaker in Molecular biology (July 2013).



New Methods and Techniques Developed/Evaluated:

Cost cutting technology, validation and documentation for wider benefit helps reduce costs of patient management; thereby increasing affordability of patients and improving adherence to medication and clinical follow up.

- In-house genotyping assay for the detection of HIV-1 drug resistance mutations for RT and PI drugs (Technology transfer from Brown University and Stanford University)
- Validation of a novel, rapid, low-cost and low-tech Microscopic Observation Drug Susceptibility (MODS) assay for detection of TB and MDR-TB.
- In-house HIV-1 cost-effective viral load assay.

Improvements in Existing Diagnostic Methods and Techniques

- Dried Blood Spots (DBS) based HIV viral load assay.
- Iodine - Glycerol method for wet mounting of faecal parasites.
- Iodine - Glycerol method for identification of fungal elements.
- DBS based assay to detect anti-HCV antibody.

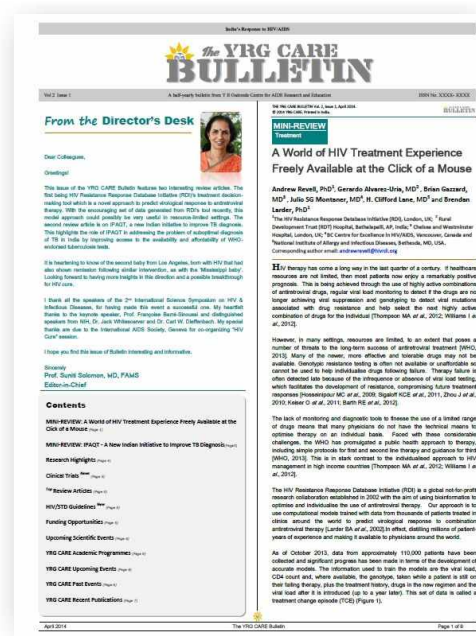
Validation of Commercial Diagnostic Kits for their Performance

- ExaVir Load assay (CAVIDI, Sweden).
- Evaluation of HIV-1 rapid and ELISA commercial (Span diagnostics, India) test kits.
- Cryptococcus Antigen detection kit (Immuno-Mycologics, Inc, Norman, USA).
- VISITECT® CD4 assay (Omega, UK).
- GeneXpert MTB/RIF assay (Cepheid, Sunnyvale, USA).

In the pipeline

- Implementation of newer international standards (ISO15189: 2012) for laboratory quality system.
- Standardization and implementation of the laboratory assays for the HIV cure research.
- Development of low-cost assay for detection of HIV drug resistance.
- Method validation of new fully automated chemistry analyzer for patient services, research and clinical trials.

The laboratory releases its periodical newsletter 'The **YRGCARE Bulletin**', featuring scientific review articles, research highlights, clinical trial news, funding opportunities and forthcoming scientific events info and useful nuggets of scientific resources, with the objective of disseminating information on recent updates in the field of HIV and infectious diseases.



Community Outreach

Bringing people and services together, the community outreach serves as an important linkage between the clinic, research unit and the people served.

YRGCARE renewed its focus on prevention and addressed the vulnerability of women in lower socio-economic groups to HIV and STDs by innovative approaches. Investments were increased to assist women living with HIV enhance their understanding about HIV and access to care.

Men as change agents in reducing domestic violence (2006-08)

Women affected by domestic violence are more likely to be at-risk for HIV and STI infections, as lack of bargaining power, economic dependence, and other cultural factors put them in compromised positions. With support from the Ford Foundation (2006-08), 150 individuals each in 15 different settings (men and women in equal numbers) identified as Community Popular Opinion Leaders (CPOs) enrolled in a training program to mobilize community awareness about domestic violence and HIV. A highly sustainable model, this initiative helped set up first response counseling centers in each such community to help women experiencing episodes of violence. Additionally each counseling center had sufficient resource lists and linkages to provide value added services as required- short stay homes, legal services offered gratis, rapport with police stations ensured interventions as needed and with Government agencies that offer support to vulnerable women. The consortium that includes NGO partners (around 40) continues sustained and comprehensive delivery of services, ranging from short stay homes for victims, paralegal aid, suicide prevention and school education for children from affected families.

Treatment Education (2006-09)

The International Treatment Preparedness Coalition (ITPC)/Tides Foundation supported a Treatment Education Audiovisual DVD in Tamil – Bouquet (Malarkothu) for those living with HIV and their care givers and friends. The treatment education was focused on ART related treatment issues and included a peer educator component for dissemination of information,

Mothers' Voices, a psychosocial program for women living with HIV (2007-08)

Mothers' Voices – "Thaiyin Kural" in Tamil, a psychosocial program for women living with HIV which started in 2006 within the clinic, expanded to the wider community with increased access to treatment education in collaboration with AIDS Project Los Angeles with support from the MAC AIDS Fund. YRGCARE took to scale its women's empowerment and support groups. By the end of 2007, 55 support groups were formed in 29 districts of Tamil Nadu, covering 1387 women living with HIV. The groups had an 80% retention rate, wherein 986 women graduated from the support group program. The support groups facilitated discussions on antiretroviral support, treatment literacy, psychosocial ramifications of infection, and economic livelihood options





Panchayat on Resisting Stigma & Homophobia (2009)

YRGCARE actively participated in the Actionplus Panchayat series on Resisting Stigma and Homophobia, held between January -April 2009 in five cities; Bangalore, Hyderabad, Chennai, Pune and Delhi. The People's Panchayat sought to understand and unravel perceptions about, struggles and accomplishments against stigma and homophobia through the voices of survivors and resisters. Through these testimonies, the Panchayat sought to address the devaluation of entire communities of people who practice alternate sexualit, to an absolute erosion of all notions of rights or dignity.

Livelihood improvement (since 2009)

YRGCARE's livelihood improvement initiatives seek to improve personal well being and public health by means of innovative and scalable economic enterprise models. These projects supplement family funds, allowing for enhanced access to healthcare, improved nutritional status, payment of tuition and school uniform and increased self worth of the entrepreneurs and their families. The Pi Project in partnership with the Johns Hopkins University helped train and establish 50 women in sex work in the production of reusable, eco-friendly bags. YRGCARE's first widely successful social entrepreneurship program, the Pi Project empowers women by providing job training, budgeting skills, awareness of critical health issues, and a sustainable income. Approximately 90% of proceeds go directly to the women who receive monthly wages with an additional income for each bag sold.

For more information, log on to www.pibags.org/bags



India's First Group Health Insurance for People Living with HIV 2006-08

YRGCARE supported the efforts of Population Services International (PSI) to introduce a specially designed insurance product of the STAR Health Insurance Co Limited, for people living with HIV in Tamil Nadu. Nearly 1000 men and women living with HIV enrolled in the plan, in various districts of Tamil Nadu. The outreach focused on the most financially vulnerable individuals. The insurance was subsequently expanded to include children living with HIV, and 1500 children were enrolled.

Training and Symposiums



Students and healthcare professionals, the pride of our programs, ensure sustainability through strengthening capacities and sharing experiences within resource limited settings

YRGCARE 's training programs and symposiums combine technical expertise, research updates while addressing local needs and requirements making it culturally appropriate as well as promoting self learning, developing skills and techniques of participants and in-house mentors and trainers.

Annual Symposiums

YRGCARE as the largest not-for-profit organization in India continues to remain a point of reference for clinicians, laboratory scientists and researchers interested in HIV service, research and management. It has strong linkages to academic institutes of excellence; national and international. This is a strategic priority for YRGCARE.



International Science Symposium on HIV & Infectious Diseases

In 2008, the laboratory initiated the first HIV Science Symposium in Chennai, co-sponsored by leading national and international research institutes. Each year, this symposium has grown to include an increasing number of national delegates from renowned universities and institutes; national and international presenters and delegates thus growing into an International Science Symposium in 2012. Abstracts published in BMC Infectious Diseases (Impact Factor 2.83). The recent 2nd International Science Symposium, on HIV & Infectious Diseases (January 30-February 1, 2014) in Chennai registered 461 delegates (21 international). Accredited by Tamilnadu Medical Council and Tamilnadu Dr. MGR Medical University which provides CME credits to participants. International faculties (25) and national faculties (14): Dr. Françoise Barré-Sinoussi, Dr. Jack Whitescarver, (Director, Office of AIDS Research National Institute of Health, USA, and Dr. Carl W. Dieffenbach, Director, Division of AIDS, National Institute of Health (NIH), USA.



Satellite Session on HIV Cure co-organized by IAS, Geneva

Satellite Symposium on “HIV Cure” was organised in conjunction with the HIV SCIENCE 2014 on 31st January 2014. Prof. Françoise (Institut Pasteur, France), **Dr. Diana Finzi** (NIH, USA), **Dr. Asier Sáez-Ciri3n** (Institut Pasteur, France) and **Dr. Anna-Laura Ross** (IAS, Geneva) presented and **Dr. Jack Whitescarver** and **Prof. Solomon** co-chaired.

Chennai ART Symposium (CART) 2014



In recent years, YRGCARE’s clinical research unit has developed several scientific networking initiatives, which serves to provide local expertise and young students with opportunities to stay up-to-date in the care and management of HIV. The 6th Chennai ART Symposium CART 2014 was held April 26-27, 2014 in Chennai, Tamil Nadu. The symposium addresses a range of topics on antiretroviral therapeutics and HIV co-infections with TB, HBV, HCV and HPV by an expert faculty of over 20 experts from various national and international academia. More than 225 physicians and researchers; ranging from governmental to non-governmental sectors participated. The symposium focuses on providing clinicians involved in managing HIV infection and HIV researchers - updates on the current concepts of antiretroviral therapy, newer drugs, their toxicities and resistance. In summary, the symposium helps put into perspective that HIV is now a chronic manageable disease; however issues of stigma and medical neglect continue to prevail. With increased efforts to meet the WHO targets of reaching to zero, the India government has merged the Department of National AIDS Control Organisation with the Health Ministry, taking "minimum government, maximum governance" to HIV prevention; thus mainstreaming HIV healthcare programs into other health ministries and departments.

HIV/AIDS Nursing Symposium (HANS)



YRGCARE conducts "HANS" in association with the M.A. Chidambaram College of Nursing over the past few years at their premises within VHS Campus, Taramani. Various HIV patient management issues pertinent to the role of nurses and midwives are presented and discussed. Participants are mainly nursing students, nurses and other nursing personnel from local colleges and hospitals.

The YRGCARE Bioethics Symposium (TYBS)



YRGCARE organises 'TYBS' – The YRGCARE Bioethics Symposium". Over the years, several collaborators have supported this initiative; the National Institute of Epidemiology, ICMR, Chennai, through the National Institutes of Health Project "Centrally coordinated Bioethics Education for India", the National Institute for Research in Tuberculosis (NIRT), ICMR, Chennai, Public Responsibility in Medicine and Research (PRIM&R), Boston, USA. The symposium explores the pressing ethical issues in medical research involving human participants and provides for an interactive dialogue and sharing of experiences of national and international professionals with local researchers and students.

GCLP Workshop on International Standards, ISO15189:2013



YRGCARE regularly organizes Good Clinical Laboratory Practice (GCLP) 3-day workshops within the city co-sponsored by the Indian Council of Medical Research (ICMR), New Delhi. The curriculum includes didactic lectures, interactive sessions, group exercises and case studies. This serves as an opportunity to professionals interested in patient care/health research, developing quality systems and in implementing comprehensive GCLP guidelines.

Educational (Updates, Internships, Fellowship)

YRGCARE provides opportunities for learning and growth to its staff through quarterly updates on clinical, behavioural and laboratory discourses related to HIV and emerging issues. Additionally, it offers short term opportunities to national and international interns and fellowship programs to work with its clinical, laboratory and behavioural science teams.

Fogarty-Ellison International Clinical Research Training Fellowship

The Fogarty International Center (FIC) of the National Institutes of Health (NIH), through the Brown-TUFTS AIDS International Training and Research Program (AITRP), awarded YRGCARE a twinning opportunity for clinical and public health graduate students from the United States and from YRGCARE. This program provides a one-year clinical research training opportunity for outstanding graduate level US students in health professions with a strong interest in international health and/or clinical research to work along with a young and promising researcher from YRGCARE. Eight(8) US and Seven(7) Indian awardees trained during this period.

2007-09

Clinical Fellowship

Through a grant from Pfizer Foundation, YRGCARE and Fenway Community Health, Boston, USA, trained 59 physicians as part of a residential fellowship.

2012

International Honors Program Team Visit

A team of US based international honors program (www.ihp.edu), on a comparative study around the world, visited YRGCARE on 18th September 2012. The students were accompanied by their faculty; Ms. Shanthi, Ms. Sara and Ms. Karen. Mrs. R. Swarnalatha, Country coordinator (Health and Community, India) co-ordinates these tours to Chennai, India each year.

2014

Continuing Medical Education

YRGCARE organized a Continuing Medical Education (CME) program on January 4, 2014 on "TB Diagnosis in the light of WHO guidelines" at Hotel Residency Towers, T. Nagar, Chennai ;

2014

The Journey of HIV Discovery

During her visit to YRGCARE, Professor Françoise Barré-Sinoussi was invited to give a public lecture on her journey of HIV discovery on January 31, 2014.



Round Table Discussion to Support Development of Public Private Partnership in Tamilnadu

Global Health Strategies (GHS) in collaboration with YRGCARE organized a round-table meeting in Chennai with civil society partners and representatives from the state government on 17th October 2012 at Hotel Residency Towers. This round-table discussion is to support the development of a public-private partnership in Tamil Nadu and bring out the need for a comprehensive approach for preventing pneumonia and diarrhea — the leading killers of children under-5.

Short Certificate Training Programs

YRGCARE promotes hands-on training and learning by doing. It capitalizes on the richness and diversity of its participants along with the expertise of in-house professionals.

Capacity Building of Avahan-supported State Lead Partners on STI/HIV Counselling Supported by Family Health International as part of Avahan Project

2007-08

To enhance the capacity of Avahan-supported State Lead Partners (SLPs) on STI/HIV counseling in order to improve the quality and coverage of counseling and referral services. YRGCARE served as key trainers and mentors.

4 Training of trainers programs; 80 Participants; Maharashtra, Madhya Pradesh, Karnataka, Mysore, Tamil Nadu, Andhra Pradesh, Nagaland and Manipur.

Assisted in 6 training workshops in other SLPs and Mentoring visits to 9 clinics

The YRGCARE core training team along with rich inputs from the state lead partners developed a set of documents: the Counseling Guidelines and Standards, a Training Toolkit for Counselors and a Monitoring and Supervisory Handbook for Quality Assurance with Quality Indicators.

2009



HIV Counseling and Testing Training for Laboratory Professionals (HTC)

YRGCARE now offers a short-term 2-day training course on Pre and Post-HIV Test Counseling for laboratory professionals. The training course is based on the National AIDS Control Organization (NACO) guidelines to ensure implementation of uniformity in counseling and testing services in private laboratories across the country. This serves the requirement of the National Accreditation Board of Laboratories (NABL) to have laboratory staff trained and certified for providing Voluntary Counseling and Testing (VCT) services. YRGCARE continues to offer longer term HIV counseling training with hands on experience.

2010-12

Women Friendly Treatment Model

Developed a Women Friendly Treatment Model to improve ART adherence for women living with HIV Locations: Andhra Pradesh and Manipur, a grant supported by International Treatment Preparedness Coalition (ITPC) and Positive Women's Network

Key highlights: Networking with healthcare providers, identifying women recently diagnosed and/or initiating ART for the first time, developing module to address key treatment issues, core issues of respect, hope, praise, being healthy and sharing with each other through women-to-women chats, providing continued support and linkages through regular meets.

Capacity Building on HIV Care, Support and Treatment for Master Trainers

2011-12

The Tata Institute of Social Sciences, Deonar, Mumbai as part of its GFATM Saksham7 Project invited YRGCARE to train a set of Master Trainers for NACO. The goal was to reorient and update knowledge and skills of Master Trainers from all over India. The Master Trainers included professionals from all over the country, involved in teaching and medical professions with a few actively involved indirect HIV public programs.

Two sets of capacity building workshops were conducted in three batches each:

> Psychosocial Issues; 58 counselors and healthcare workers (2011) and

>> Clinical Issues; 47 physicians and counselors (2011-12)

The training modules for Clinical and Psychosocial Issues are available on request.

Capacity Building Training on Effective Programme Implementation

2011-12

Supported by DFID AIDS Technical Assistance Support Team and Futures Group International India Pvt. Ltd. for NACO Under the Short Term Technical Assistance Training to Grantees of Innovation Fund.

Locations: Mumbai, Manipur, Chennai and Kolkata.

Positive Prevention: Moving Beyond

2011-12

A curriculum developed and piloted by YRGCARE st for Andhra Pradesh State AIDS Control Society (APSACS) as part of their extension on Targeted Interventions Program to work with persons with high risk behavior who are living with HIV (NACP III)

2013-14

Regional Capacity Building Program on Child Centric Techniques

Strengthening Capacity of healthcare workers and students interested in working with pediatrics and adolescents: Much of HIV prevention, advocacy and support starts with training of like-minded persons to provide better care services. There are separate leadership structures, support teams, schedules and roles and responsibilities. However, in the real world, the borders blur. The same issues arise. It's essential to merge some of these realities with the newness of the programs that we build and expand our programs to be child-centric and adolescent friendly. We identified some key concepts in child-centric techniques to take our programs to scale. These were mainly supported by the SeriousFun Children's Network with additional financial support from the MAC AIDS Fund.

Between April 2013-Sept 2014, over 188 key personnel from various organizations serving children directly through their HIV and other related support programs including students from leading colleges and universities were trained on child centric techniques with the additional hands on learning of a one-day camp or a 5-day residential camp experience where adult learners had the opportunity to take their learning into practice directly in a safe environment. These adults hail from the metros of Bangalore and Chennai and neighbouring areas as well as from other large districts of Madurai, Theni and Dindigul. Current Partnerships: CHES, ASHA Foundation, SeriousFun Children's Network.

Read: www.yrgcare.org/ipi

Enhancing Community Opportunities (ECO)

The Foundation's Project ECO –
“Enhancing Community Opportunities”
facilitates economic stability within families
affected by social and economic challenges,
and strives to address inequity through
opportunities.

History

The work of the Foundation in Chennai communities between 2001 and 2008 highlighted the issues faced by them: intimate partner violence, increasing school dropout rates, spousal issues of alcoholism, financial instability, unmet childcare, household and health needs.

At its core was the economic dependence of women on their partners. Project ECO (Enhancing Community Opportunities) was initiated in 2009 to address financial stability within families affected by social, economic or other challenges and provide women with a stable income.

Project ECO Livelihood Support Initiatives



The Foundation's Project ECO – “Enhancing Community Opportunities” has two key initiatives; ‘Pibags’ and ‘ECO Kitchen’ that facilitates economic stability within families affected by social and economic challenges and strives to address inequity through opportunities.



A Bag Full of Hope

Women seeking autonomy after facing violence, poverty and hunger find a world of hope through Pi Bags. This initiative provides valuable work and entrepreneurial skills and education to marginalized women in Chennai and offers them economic empowerment through stitching of eco-friendly bags. Women graduate from an intensive 100-hour bag-making training by master tailors and exercise a choice to either establish their own business or join other women at Pi Bags to produce bags and accessories sold throughout the world. Since 2009, this initiative has supported 60 pibags entrepreneurs.



Pi bags has no retail outlet. Sales are realized through word-of-mouth. These products are sought for conferences, weddings, reunions etc. Toned with lovely hues in canvas and jute material, 100% of the profits of the Pi bags sales go to the entrepreneurs. Till date more than 20,000 bags have been produced and sold.

An innovative and scalable economic enterprise, these women entrepreneurs are now able to supplement family funds for enhanced health care, improved nutritional status, payment of tuition and school uniforms and most importantly increased self worth.

ECO KITCHEN

Delightfully Vegetarian



18,000 sq. feet green facility

*offers delicious, healthy meals
and snacks, including
organic options*

*capacity to serve over
30000 meals per meal time*

*uses innovative,
eco-conscious technologies*

promotes social entrepreneurship

supports communities in chennai

Vision

Our vision is that our meals will nourish bodies and empower minds to become more confident, capable, and proud.



ECO Kitchen was inaugurated on October 25, 2010.

FACILITY

ECO Kitchen is an 18000 sq. ft. facility with a 30,000 meal production capacity, a scale that assures low production costs. The kitchen is a green, naturally lit and ventilated building which utilizes innovative, eco-conscious technology including bio-mass briquettes, solar geysers, steam condensation recovery system, expansive drainage system and a wastewater treatment plant that irrigates the garden. These production systems have been thus crafted to minimize impact upon the environment.

It is a FSSAI certified facility.



Partnering with SODEXO, a global leader in food services, through their CSR initiative "Better Tomorrow Plan", ensures that each meal is safe, healthy and delicious. Sodexo's Better Tomorrow Plan, supports initiatives that protect the environment, promote health and well-being, and support the development of communities.

"It was a proud moment for us when we received the Subir Raha Corporate Social Responsibility Awards 2012 for Community Development, in partnership with ECO Kitchen, as recognition for our efforts".

- Sunil Nayak, CEO, Sodexo India On-Site Service Solutions.



ECO Kitchen offers delicious, vegetarian meals of outstanding value. More than just a tantalizing treat, each ECO Kitchen meal helps to improve the lives of underserved families in Chennai. ECO Kitchen maintains its health, safety and regulatory standards and compliance.

ECO Friendly Technologies



VENTILATED FACILITY



SOLAR WATER HEATER



BIOMASS BRIQUETTES



EMPOWERING LOCAL COMMUNITIES



HIGH ENERGY STEAM VATS



BIOMASS COOK STOVES

ECO Kitchen has successfully complemented its growth through three key components



Kathir: innovative food retail micro-entrepreneurship supporting disadvantaged women and their families to realize their socioeconomic potential. This initiative assists women, who are victims of domestic violence, spouses of drug users, or caregivers of families living with HIV or other chronic diseases with opportunities to improve their livelihoods through a food retail programme. This programme makes available affordable and healthy food in the city of Chennai.

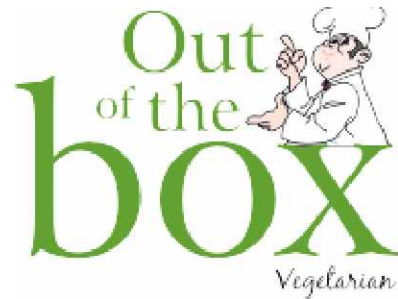
Since 2009, Kathir has trained 454 women entrepreneurs and provides on an average 1500-1800 meals per day at 88 kiosks in South Chennai within a radius of 15 kms from the kitchen. Vending kiosks are provided to the women free of cost to reduce their burden upfront and enhance their ability to try out their newly developed skills for food retail sales. These costs are supported by Rotary Clubs from Chennai, India and Rockville, USA, through the Global Giving portal, Kindness in Action, USA and corporate philanthropists.



A bridge between those in need and those who wish to help.

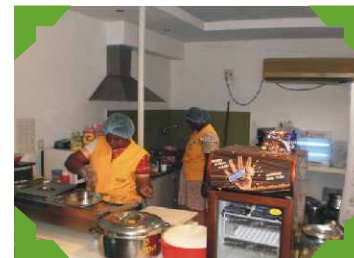
This initiative began in 2010 to serve meals to those most in need: children, women, elderly and differently abled in orphanages, destitute homes and poorer communities of Chennai. Meals are nutritious and hot, prepared with fresh and high quality ingredients and produced at scale. Kathir Dhan presently serves over 70 institutions in Chennai.





Out-of-the-Box serves personalized meals, corporate pantries and private catering events. "Out of the Box" serves a variety of meals including organic options, heritage meals and snacks made from millets.

The program offers job opportunities to marginalised women by providing training on food handling, self confidence, grooming, and personal hygiene. Currently we have 163 food service agents, at 34 corporate client eateries and 400 private catering events.



ECO Kitchen and Sodexo jointly sensitize hundreds of female workers at their Nokia facility, Chennai

During 2011-12, trainers from ECO Kitchen partnered with Sodexo to train female employees in personal hygiene, grooming, self confidence, gender awareness and psychosocial well being.

Project ECO is grateful for the support from corporate and community volunteers through whom it connects those in need in the community with Government and other non Government service providers, and enables improved reproductive health and child development.

World Food Day 2012

From Hunger to Hope

On World Food Day 2012, over 500 volunteers from academic and corporate institutes along with 70 Sodexo staff helped serve more than 30,000 people in Chennai city (12 community sites, 21 institutions)

The average number of meals continues to increase with support of trained vendors, the local communities served and the supporting staff. This event was organized in partnership with Sodexo and Queen Mary's College, Chennai.



**Marking the induction of twenty five new entrepreneurs,
a gift of Rotary Clubs of Chennai and Rockville, USA.**

World AIDS Day



International Women's Day 2012

International Women's Day 2013

Towards Zero

ECO Kitchen conducted a quiz competition (fashioned in the famed Television format KBC 'Kaun Bannega Crorepati') for the school children at Nungambakkam Corporation School to infuse fun into learning about HIV. Special focus was on HIV prevention and elimination of stigma thereby providing an enhancing and supportive environment for individuals and families affected.

Chennai Inspires

Women's Christian College (WCC), Chennai partnered to organize a meeting for their students at the WCC Auditorium on 08th March 2012. This has turned into an annual event.

Society for the Upliftment of the Economically Backward (SUEB), Chennai, caters to women's development and vocational programs, in southern Chennai. On the event of International Women's Day 2013, SUEB collaborated with ECO Kitchen to provide micro entrepreneurship programs for marginalised women. Three hundred (300) women from neighbouring communities participated to celebrate Women's Livelihood.

Project ECO has another component: Endrum Punnagai



Endrum Punnagai helps children at schools (Corporation of Chennai) to enhance health, self-confidence and interpersonal communication. A programme for boys and girls of disadvantaged families that uses active learning (games, role play, group learning etc) to foster enquiry and facilitate expressions. Delivered over several sessions, once per week at the schools, it helps build self-confidence, self-awareness, healthy perspectives and an enhanced sense of community. It assists boys and girls to understand risks. The culminating activity occurs at the ECO Kitchen over a custom cooked hot meal and a discovery tour.



Since 2011, more than 900 adolescent girls and boys from Corporation of Chennai and Government schools in Chennai city have been part of interactive participatory and thought provoking sessions. “Endrum Punnagai” develops confidence, enhances life skills and relationship building sessions; focusing on personal hygiene, nutrition and reproductive health. Students discover a range of eco conscious technologies and understand keys to nutritional wellness. In this group learning activity, the participants develop bonding and connect with a world beyond school and home. A definite expansion is to include more schools into the program with the concept of activity based learning, exploration and fun with the support of volunteers.

Nutrition Discovery Tour



Open to students from academic institutions; from kindergarten to college, discovery tours help students to understand the processes at ECO kitchen and share a nutritious and delicious meal with staff and volunteers.

The core aspect of the discovery tour is to ensure that the young visitors have fun! They are engaged in interactive sessions; quizzes on nutrition and life skills, photo sessions, etc.

The ECO Kitchen team who pose as tour guides, look forward to such exchanges; note their feedback especially from the youngest of visitors and make improvements.

Strategic Partnerships

The Y.R.Gaitonde Medical Education and Research Foundation engages public and private partnerships on all levels, from local community groups to multinational leaders in public health. Our partners include the following:

National Organisations/Institutes/Universities

Academic Institutes from Chennai city
Actionplus
Anna University
ARCON
ASHA Foundation, Bangalore
Asha Kirana, Mysore
Chennai Corporation
CHUM Society, Mirialguda, AP
Community Health Education Society (CHES), Chennai
Cognizant Technology Solutions
DESIRE, Medhak, AP
Family Health International (FHI)
Federation of Indian Chambers of Commerce and industry (FICCI)
Ford Foundation
Freedom Foundation, Bangalore and Hyderabad

National Organisations/Institutes/Universities

The French Institute of Pondicherry and Group of Research Innovations and Societies
GRIS, Department of Sociology, University of Rouen, France
Karnataka Health Promotion Trust (KHPT), Bangalore
KIMS, Bangalore
IBM - Digital Literacy and Volunteer Programs
Indian Red Cross, Nellore, AP
International AIDS Vaccine Initiative (IAVI)
Indian Council of Medical Research (ICMR), New Delhi
Kindness in Action
National AIDS Research Institute (NARI), Chennai
Pfizer
Post Graduate Institute of Medical Education and Research (PGIMER), Chandigarh
Population Council, New Delhi
Population Services International (PSI)
Raagas Dental College, Chennai
Rotary Club, Madras Mid Town
Sankara Nethralaya, Chennai
Sun Foundation, Chennai
St. Ann's Hospital, Vizag
Tuberculosis Research Center (TRC) , Chennai
United Way Chennai
University of Madras, Chennai

International Institutes / Universities

Adult AIDS Clinical Trial Group (AACTG), National Institutes of Health (NIH), USA
AIDS Project Los Angeles, CA, USA
American India Foundation (AIF)
American Foundation for AIDS Research (AMFAR)
Andrew Zeigler Foundation, SF, USA
Brown CFAR, RI, USA
Burnet Institute for Medical Research, Melbourne, Australia
Centers for Disease Control, Atlanta, USA
Centre for AIDS Prevention Studies, University of California, SF, CA, USA
Chicago Medical School, USA
Contraceptive Research And Development Program (CONRAD) and Family Health International, USA
Copenhagen Business School
Division of AIDS (DAIDS), USA
Duke University, CA, USA
Emory University, Atlanta, USA
Fogarty International Center, NIH, USA
Global Fund to Fight against AIDS, TB and Malaria (GFATM)
Global Giving
Gladstone Institute of Immunology, CA, USA
HIV Prevention Trials Network (HPTN), National Institutes of Health (NIH), USA
Horizons
Infectious Diseases Research Laboratory, UPCH, Lima, Peru
International HIV/AIDS Alliance
Johns Hopkins Bloomberg School of Public Health, USA
Karolinska Institute, Sweden

International Institutes / Universities

Lifespan Institutes

MAC AIDS Fund, USA

MacFarlane Burnet Institute for Medical Research and Public Health, Australia

Miriam Hospital, Brown University, The Division of Infectious Diseases, RI, USA

National Cancer Institute, USA, AIDS Malignancy Consortium

National Institutes of Health, USA

National Institutes of Mental Health (NIMH)

New Jersey School of Medicine, Newark, USA

Population Council

Prince Leopold Institute of Tropical Medicine, Antwerp, Belgium

Rotary Clubs of Rockville

Rush University, Chicago, USA

Scottish Centre for Infectious and Environmental Health (SCIEH), Glasgow, UK

San Diego State University (SDSU), USA

SeriousFun Children's Network, Connecticut, USA

Sodexo

Stanford University, CA, USA

Treat Asia HIV Observational Database (TAHOD)

University of California, Davis, USA

University of California, San Diego (UCSD), USA

University Of Copenhagen (Nutrition and Entrepreneurship)

US Centers for Disease Control and Prevention (CDC)

Yale School of Public Health, Yale University, USA

World AIDS Foundation (WAF), USA

World Health Organization (WHO)

Journal Publications (2007-2014)

2007

Financial burden of health services for people with HIV/AIDS in India.

Kumarasamy N, Venkatesh KK, Mayer KH, Freedberg K. Indian J Med Res. 2007 Dec;126(6):509-17. Review.

PMID: 18219077

Ethnic variation in certain hematological and biochemical reference intervals in a south Indian healthy adult population.

Sundaram M, Mohanakrishnan J, Murugavel KG, Shankar EM, Solomon S, Srinivas CN, Solomon SS, Pulimi S, Piwowar-Manning E, Dawson S, Livant E, Kumarasamy N, Balakrishnan P. Eur J Intern Med. 2008 Jan;19(1):46-50. doi: 10.1016/j.ejim.2007.06.010. Epub 2007 Oct 26.

PMID: 18206601

Immune reconstitution inflammatory syndrome in association with HIV/AIDS and tuberculosis: views over hidden possibilities.

Shankar EM, Vignesh R, Murugavel KG, Balakrishnan P, Sekar R, Lloyd CA, Solomon S, Kumarasamy N. AIDS Res Ther. 2007 Nov 30;4:29.

PMID: 18053126

Use of an HIV-1 reverse-transcriptase enzyme-activity assay to measure HIV-1 viral load as a potential alternative to nucleic acid-based assay for monitoring antiretroviral therapy in resource-limited settings.

Iqbal HS, Balakrishnan P, Cecelia AJ, Solomon S, Kumarasamy N, Madhavan V, Murugavel KG, Ganesh AK, Solomon SS, Mayer KH, Crowe SM. J Med Microbiol. 2007 Dec;56(Pt 12):1611-4.

PMID: 18033828

High proportion of isosporiasis among HIV-infected patients with diarrhea in southern India.

Vignesh R, Balakrishnan P, Shankar EM, Murugavel KG, Hanas S, Cecelia AJ, Thyagarajan SP, Solomon S, Kumarasamy N. Am J Trop Med Hyg. 2007 Nov;77(5):823-4.

PMID: 17984334

Coinfection of hepatitis B and hepatitis C virus in HIV-infected patients in south India.

Saravanan S, Velu V, Kumarasamy N, Nandakumar S, Murugavel KG, Balakrishnan P, Suniti S, Thyagarajan SP. World J Gastroenterol. 2007 Oct 7;13(37):5015-20.

PMID: 17854146

Sexual behaviors of individuals with HIV living in South India: a qualitative study.

Sri Krishnan AK, Hendriksen E, Vallabhaneni S, Johnson SL, Raminani S, Kumarasamy N, Hobsen J, Solomon S, Mayer KH, Safren SA. AIDS Educ Prev. 2007 Aug;19(4):334-45.

PMID: 17685846

A randomised control trial of structured interrupted generic antiretroviral therapy versus continuous therapy in HIV-infected individuals in Southern India.

Kumarasamy N, Flanigan TP, Vallabhaneni S, Cecelia AJ, Christybai P, Balakrishnan P, Yephthomi T, Solomon S, Carpenter CC, Mayer KH. AIDS Care. 2007 Apr;19(4):507-13.

PMID: 17453591

2008

Gender-based differences in treatment and outcome among HIV patients in South India.

Kumarasamy N, Venkatesh KK, Cecelia AJ, Devaleenol B, Saghayam S, Yephthomi T, Balakrishnan P, Flanigan T, Solomon S, Mayer KH. J Womens Health (Larchmt). 2008 Nov;17(9):1471-5. doi: 10.1089/jwh.2007.0670.

PMID: 18954236

Expansion of HIV laboratory diagnostic services in Chennai, India 2001-2006: is the growth commensurate with the need?

Srikrishnan AK, Venkatesh KK, Solomon S, Thamburaj E, Anand S, Kosalaraman KG, Balakrishnan P, Mayer KH. PLoS One. 2008;3(10):e3471. doi: 10.1371/journal.pone.0003471. Epub 2008 Oct 21.

PMID: 18941613

Regression of Kaposi's sarcoma lesions following highly active antiretroviral therapy in an HIV-infected patient.

Kumarasamy N, Venkatesh KK, Devaleenol B, Poongulali S, Ahilasamy N. Int J STD AIDS. 2008 Nov;19(11):786-8. doi: 10.1258/ijsa.2008.008016.

PMID: 18931277

Urinary infections due to multi-drug-resistant Escherichia coli among persons with HIV disease at a tertiary AIDS care centre in South India.

Vignesh R, Shankar EM, Murugavel KG, Kumarasamy N, Sekar R, Irene P, Solomon S, Balakrishnan P. Nephron Clin Pract. 2008;110(1):c55-7. doi: 10.1159/000151533. Epub 2008 Aug 25.

PMID: 18724067

Alpha-fetoprotein as a tumor marker in hepatocellular carcinoma: investigations in south Indian subjects with hepatotropic virus and aflatoxin etiologies.

Murugavel KG, Mathews S, Jayanthi V, Shankar EM, Hari R, Surendran R, Vengatesan A, Raghuram K, Rajasambandam P, Murali A, Srinivas U, Palaniswamy KR, Pugazhendhi T, Thyagarajan SP. Int J Infect Dis. 2008 Nov;12(6):e71-6. doi: 10.1016/j.ijid.2008.04.010. Epub 2008 Jul 26.

PMID: 18658001

Comorbidities among HIV-infected injection drug users in Chennai, India.

Solomon SS, Hawcroft CS, Narasimhan P, Subbaraman R, Srikrishnan AK, Cecelia AJ, Suresh Kumar M, Solomon S, Gallant JE, Celentano DD. Indian J Med Res. 2008 May;127(5):447-52.

PMID: 18653907

Prevalence and incidence of sexually transmitted infections among South Indians at increased risk of HIV infection.

Kumarasamy N, Balakrishnan P, Venkatesh KK, Srikrishnan AK, Cecelia AJ, Thamburaj E, Solomon S, Mayer KH. AIDS Patient Care STDS. 2008 Aug;22(8):677-82. doi: 10.1089/apc.2007.0166.

PMID: 18627276

Changes in antioxidant profile among HIV-infected individuals on generic highly active antiretroviral therapy in southern India.

Sundaram M, Saghayam S, Priya B, Venkatesh KK, Balakrishnan P, Shankar EM, Murugavel KG, Solomon S, Kumarasamy N. Int J Infect Dis. 2008 Nov;12(6):e61-6. doi: 10.1016/j.ijid.2008.04.004. Epub 2008 Jul 14.

PMID: 18621564

Co-factors for abnormal lactate levels among persons with HIV disease at a tertiary HIV care setting in South India.

Sundaram M, Srinivas CN, Shankar EM, Deepak M, Murugavel KG, Balakrishnan P, Solomon S, Kumarasamy N. Food Chem Toxicol. 2008 Aug;46(8):2823-5. doi: 10.1016/j.fct.2008.05.015. Epub 2008 May 24.

PMID: 18586371

GB virus infection: a silent anti-HIV panacea within?

Shankar EM, Solomon SS, Vignesh R, Murugavel KG, Sundaram M, Solomon S, Balakrishnan P, Kumarasamy N. Trans R Soc Trop Med Hyg. 2008 Dec;102(12):1176-80. doi: 10.1016/j.trstmh.2008.04.034. Epub 2008 May 29. Review.

PMID: 18513775

Ramsay hunt syndrome in a person with HIV disease.

Devaleenal DB, Ahilasamy N, Solomon S, Kumarasamy N. Indian J Otolaryngol Head Neck Surg. 2008 Jun;60(2):171-3. doi: 10.1007/s12070-008-0041-x. Epub 2008 Jun 12.

PMID: 23120531

Performance characteristics of a new rapid immunochromatographic test for the detection of antibodies to human immunodeficiency virus (HIV) types 1 and 2.

Syed Iqbal H, Balakrishnan P, Murugavel KG, Suniti S. J Clin Lab Anal. 2008;22(3):178-85. doi: 10.1002/jcla.20238.

PMID: 17984334

Spectrum of adverse events after generic HAART in southern Indian HIV-infected patients.

Kumarasamy N, Venkatesh KK, Cecelia AJ, Devaleenal B, Lai AR, Saghayam S, Balakrishnan P, Yepthomi T, Poongulali S, Flanigan TP, Solomon S, Mayer KH. AIDS Patient Care STDS. 2008 Apr;22(4):337-44. doi: 10.1089/apc.2007.0093.

PMID: 18422462

Atypically distributed cutaneous lesions of Norwegian scabies in an HIV-positive man in South India: a case report.

Ramachandran V, Shankar EM, Devaleenal B, Pachamuthu B, Thousen SM, Sekar R, Suniti S, Nagalingeswaran K. J Med Case Rep. 2008 Mar 14;2:82. doi: 10.1186/1752-1947-2-82.

PMID: 18341688

Does CD4+CD25+foxp3+ cell (Treg) and IL-10 profile determine susceptibility to immune reconstitution inflammatory syndrome (IRIS) in HIV disease?

Shankar EM, Vignesh R, Velu V, Murugavel KG, Sekar R, Balakrishnan P, Lloyd CA, Saravanan S, Solomon S, Kumarasamy N. J Inflamm (Lond). 2008 Feb 18;5:2. doi: 10.1186/1476-9255-5-2.

PMID: 18282273

2009

Genotypic HIV type-1 drug resistance among patients with immunological failure to first-line antiretroviral therapy in south India.

Vidya M, Saravanan S, Uma S, Kumarasamy N, Sunil SS, Kantor R, Katzenstein D, Ramratnam B, Mayer KH, Suniti S, Balakrishnan P. Antivir Ther. 2009;14(7):1005-9. doi: 10.3851/IMP1411.

PMID: 19918105

Risk factors for HIV transmission among heterosexual discordant couples in South India.

Kumarasamy N, Venkatesh KK, Srikrishnan AK, Prasad L, Balakrishnan P, Thamburaj E, Sharma J, Solomon S, Mayer K. HIV Med. 2010 Mar;11(3):178-86. doi: 10.1111/j.1468-1293.2009.00760.x. Epub 2009 Sep 24.

PMID: 19780862

Primary HIV-1 drug resistance and polymorphic patterns among injecting drug users (IDUs) in Chennai, Southern India.

Iqbal HS, Solomon SS, Madhavan V, Solomon S, Balakrishnan P. J Int Assoc Physicians AIDS Care (Chic). 2009 Sep-Oct;8(5):323-7. doi: 10.1177/1545109709341854. Epub 2009 Aug 31.

PMID: 19721100

Common protozoans as an uncommon cause of respiratory ailments in HIV-associated immunodeficiency.

Shankar EM, Vignesh R, Murugavel KG, Balakrishnan P, Ponmalar E, Rao UA, Velu V, Solomon S. FEMS Immunol Med Microbiol. 2009 Nov;57(2):93-103. doi: 10.1111/j.1574-695X.2009.00588.x. Epub 2009 Jul 24. Review.

PMID: 19702877

Factors associated with mortality among HIV-infected patients in the era of highly active antiretroviral therapy in southern India.

Kumarasamy N, Venkatesh KK, Devaleenol B, Poongulali S, Yephthomi T, Pradeep A, Saghayam S, Flanigan T, Mayer KH, Solomon S. Int J Infect Dis. 2010 Feb;14(2):e127-31. doi: 10.1016/j.ijid.2009.03.034. Epub 2009 Jul 25.

PMID: 19632872

High frequency of clinically significant mutations after first-line generic highly active antiretroviral therapy failure: implications for second-line options in resource-limited settings.

Kumarasamy N, Madhavan V, Venkatesh KK, Saravanan S, Kantor R, Balakrishnan P, Devaleenol B, Poongulali S, Yephthomi T, Solomon S, Mayer KH, Benson C, Schooley R.

Clin Infect Dis. 2009 Jul 15;49(2):306-9. doi: 10.1086/600044.

PMID: 19522657

Evaluation of two human immunodeficiency virus-1 genotyping systems: ViroSeq 2.0 and an in-house method.

Saravanan S, Vidya M, Balakrishnan P, Kumarasamy N, Solomon SS, Solomon S, Kantor R, Katzenstein D, Ramratnam B, Mayer KH. J Virol Methods. 2009 Aug;159(2):211-6. doi: 10.1016/j.jviromet.2009.03.021. Epub 2009 Apr 2. Erratum in: J Virol Methods. 2010 Feb;163(2):513. Balakrishnan, P [corrected to Balakrishnan, P].

PMID: 19490976

A longitudinal quality-of-life study of HIV-infected persons in South India: the case for comprehensive clinical care and support services.

Solomon S, Batavia A, Venkatesh KK, Brown L, Verma P, Cecelia AJ, Daly C, Mahendra VS, Kumarasamy N, Mayer KH. AIDS Educ Prev. 2009 Apr;21(2):104-12. doi: 10.1521/aeap.2009.21.2.104.

PMID: 19397433

Safe substitution to zidovudine among HIV-infected patients initiated on stavudine-containing highly active antiretroviral therapy from a resource-limited setting.

Kumarasamy N, Venkatesh KK, Devaleenol B, Saghayam S, Manohar D, Poongulali S, Yepthomi T, Ambrose P, Solomon S, Mayer KH. Int J Infect Dis. 2009 Nov;13(6):e360-4. doi: 10.1016/j.ijid.2008.11.030. Epub 2009 Mar 27.

PMID: 19328734

Establishment of T-lymphocyte subset reference intervals in a healthy adult population in Chennai, India.

Murugavel KG, Balakrishnan P, Mohanakrishnan J, Solomon SS, Shankar EM, Muthu Sundaram SP, Kumarasamy N, Piwowar-Manning E, Livant E, Mayer KH, Thyagarajan SP, Solomon S. Indian J Med Res. 2009 Jan;129(1):59-63.

PMID: 19287058

Cofactors for low serum albumin levels among HIV-infected individuals in Southern India.

Sundaram M, Srinivas CN, Shankar EM, Balakrishnan P, Solomon S, Kumarasamy N.

J Int Assoc Physicians AIDS Care (Chic). 2009 May-Jun;8(3):161-4. doi: 10.1177/1545109708330787. Epub 2009 Jan 27.

PMID: 19176315

Safety, tolerability and effectiveness of generic HAART in HIV-infected children in South India.

Kumarasamy N, Venkatesh KK, Devaleenol B, Poongulali S, Mothi SN, Solomon S.

J Trop Pediatr. 2009 Jun;55(3):155-9. doi: 10.1093/tropej/fmn080. Epub 2008 Oct 1.

PMID: 17984334

2010

Analysis of selection pressure and mutational pattern of HIV type 1 reverse transcriptase region among treated and nontreated patients.

Shanmugasundaram U, Solomon S, Madhavan V, Kailapuri MG, Nagalingeswaran K, Solomon SS, Mayer KH, Pachamuthu B. AIDS Res Hum Retroviruses. 2010 Oct;26(10):1093-6. doi: 10.1089/aid.2009.0300. Epub 2010 Sep 19.

PMID: 20849301

Orofacial and systemic manifestations in 212 paediatric HIV patients from Chennai, South India.

Ranganathan K, Geethalakshmi E, Krishna Mohan Rao U, Vidya KM, Kumarasamy N, Solomon S. Int J Paediatr Dent. 2010 Jul;20(4):276-82. doi: 10.1111/j.1365-263X.2010.01050.x.

PMID: 20536589

Couples at risk for HIV infection in Southern India: characteristics of HIV-infected patients in concordant and discordant heterosexual relationships.

Kumarasamy N, Venkatesh KK, Srikrishnan AK, Prasad L, Balakrishnan P, Murugavel KG, Thamburaj E, Solomon S, Mayer KH. Int J STD AIDS. 2010 Feb;21(2):96-100. doi: 10.1258/ijsa.2008.008418.

PMID: 20089993

Can ionic imbalance in HIV disease be attributed to certain underlying opportunistic infections?

Sundaram M, Shankar EM, Srinivas CN, Solomon S, Balakrishnan P, Kumarasamy N.

Indian J Clin Biochem. 2010 Jan;25(1):105-7. doi: 10.1007/s12291-010-0002-5. Epub 2010 Feb 10. No abstract available.

PMID: 23105894

2011

Low-cost assays for monitoring HIV infected individuals in resource-limited settings.

Balakrishnan P, Iqbal HS, Shanmugham S, Mohanakrishnan J, Solomon SS, Mayer KH, Solomon S. Indian J Med Res. 2011 Dec;134(6):823-34. doi: 10.4103/0971-5916.92628. Review.

PMID: 22310816

Antiretroviral therapy in Indian setting: when & what to start with, when & what to switch to?

Kumarasamy N, Patel A, Pujari S. Indian J Med Res. 2011 Dec;134(6):787-800. doi: 10.4103/0971-5916.92626. Review.

PMID: 22310814

Whole-mount in situ hybridization of sectioned tissues of species hybrids to detect cis-regulatory changes in gene expression pattern.

Futahashi R. Methods Mol Biol. 2011;772:319-28. doi: 10.1007/978-1-61779-228-1_19.

PMID: 22065447

HIV-1 drug resistance among newly HIV-1 infected individuals attending tertiary referral center in Chennai, India.

Iqbal HS, Solomon SS, Saravanan S, Vidya M, Kumarasamy N, Solomon S, Balakrishnan P. Indian J Med Sci. 2011 Nov;65(11):488-96. doi: 10.4103/0019-5359.109538.

PMID: 23525026

Current Views on the Pathophysiology of GB Virus C Coinfection with HIV-1 Infection.

Shankar EM, Balakrishnan P, Vignesh R, Velu V, Jayakumar P, Solomon S. Curr Infect Dis Rep. 2011 Feb;13(1):47-52. doi: 10.1007/s11908-010-0142-z.

PMID: 21308454

Safety, Tolerability, and Efficacy of Second-Line Generic Protease Inhibitor Containing HAART after First-Line Failure among South Indian HIV-Infected Patients.

Kumarasamy N, Venkatesh KK, Devaleenal B, Poongulali S, Yepthomi T, Solomon S, Flanigan TP, Mayer KH. J Int Assoc Physicians AIDS Care (Chic). 2011 Mar-Apr;10(2):71-5. doi: 10.1177/1545109710382780. Epub 2011 Jan 24.

PMID: 21266320

2012

Gender differences in oral lesions among persons with HIV disease in Southern India.

Rao UK, Ranganathan K, Kumarasamy N. J Oral Maxillofac Pathol. 2012 Sep;16(3):388-94. doi: 10.4103/0973-029X.102492.

PMID: 23248472

Evaluation of two indigenous rapid and two ELISA assays for the diagnosis of HIV infection India.

Iqbal HS, Solomon S, Murugavel KG, Solomon SS, Balakrishnan P. Indian J Med Microbiol. 2012 Oct-Dec;30(4):397-402. doi: 10.4103/0255-0857.103758.

PMID: 23183462

Clinical outcomes among HIV/tuberculosis-coinfected patients developing immune reconstitution inflammatory syndrome after HAART initiation in South India.

Kumarasamy N, Venkatesh KK, Vignesh R, Devaleenal B, Poongulali S, Yeptthomi T, Flanigan TP, Benson C, Mayer KH. J Int Assoc Provid AIDS Care. 2013 Jan-Feb;12(1):28-31. doi: 10.1177/1545109712457711. Epub 2012 Sep 24.

PMID: 23011868

Polymorphisms of HIV RT Gene Among the ART Naïve Native Drug Exposed Rural PLHA.

Krishnan KM, Amsavathani S. J Glob Infect Dis. 2012 Apr;4(2):110-3. doi: 10.4103/0974-777X.96775.

PMID:22754246

Unusual insertion and deletion at codon 67 and 69 of HIV type 1 subtype C reverse transcriptase among first-line highly active antiretroviral treatment-failing South Indian patients: association with other resistance mutations.

Saravanan S, Madhavan V, Kantor R, Sivamalar S, Gomathi S, Solomon SS, Kumarasamy N, Smith DM, Schooley RT, Solomon S, Balakrishnan P. AIDS Res Hum Retroviruses. 2012 Dec;28(12):1763-5. doi: 10.1089/AID.2011.0331. Epub 2012 May 21.

PMID: 22404052

Dried blood spots versus plasma for the quantitation of HIV-1 RNA using a real-Time PCR, m2000rt assay.

Vidya M, Saravanan S, Rifkin S, Solomon SS, Waldrop G, Mayer KH, Solomon S, Balakrishnan P. J Virol Methods. 2012 May;181(2):177-81. doi: 10.1016/j.jviromet.2012.02.006. Epub 2012 Mar 1.

PMID: 22401801

Viremia and HIV-1 drug resistance mutations among patients receiving second-line highly active antiretroviral therapy in Chennai, Southern India.

Saravanan S, Vidya M, Balakrishnan P, Kantor R, Solomon SS, Katzenstein D, Kumarasamy N, Yeptomi T, Sivamalar S, Rifkin S, Mayer KH, Solomon S. Clin Infect Dis. 2012 Apr;54(7):995-1000. doi: 10.1093/cid/cir967. Epub 2012 Feb 9.

PMID: 22323567

2013

Experiences in recruiting volunteers through community based initiatives in phase-1 vaccine trials in India.

Sahay S, Kumar M, Srikrishnan AK, Ramanathan V, Mehendale S.

Hum Vaccin Immunother. 2014;10(2):485-91. doi: 10.4161/hv.26799. Epub 2013 Oct 18. Review.

PMID: 24141176

Beyond first-line HIV treatment regimens: the current state of antiretroviral regimens, viral load monitoring, and resistance testing in resource-limited settings.

Kumarasamy N, Krishnan S. Curr Opin HIV AIDS. 2013 Nov;8(6):586-90. doi: 10.1097/

COH. 0000000000000004. Review.

PMID: 24100872

A rapid and low-cost microscopic observation drug susceptibility assay for detecting TB and MDR-TB among individuals infected by HIV in South India.

Solomon S, Balakrishnan P, Vignesh R, Waldrop G, Solomon SS, Murugavel KG, Kumarasamy N, Yepthomi T, Poongulali S, Swathirajan CR, Sreenivasan V, Chandrasekar C, Suriakumar J, Mahilmaran A, Manoharan G, Moore DA. Indian J Med Microbiol. 2013 Apr-Jun;31(2):130-7. doi: 10.4103/0255-0857.115225.

PMID: 23867668

Impact of highly active antiretroviral therapy on salivary flow in patients with human-immuno deficiency virus disease in Southern India.

Pavithra S, Ranganathan K, Rao UK, Joshua E, Rooban T, Kumarasamy N.
J Oral Maxillofac Pathol. 2013 Jan;17(1):17-22. doi: 10.4103/0973-029X.110695.

PMID: 23798824

Darunavir is a good third-line antiretroviral agent for HIV type 1-infected patients failing second-line protease inhibitor-based regimens in South India.

Saravanan S, Madhavan V, Balakrishnan P, Smith DM, Solomon SS, Sivamalar S, Poongulali S, Kumarasamy N, Schooley RT, Solomon S, Kantor R. AIDS Res Hum Retroviruses. 2013 Mar;29(3):630-2. doi: 10.1089/AID.2011.0334. Epub 2012 Dec 7.

PMID: 2304596

2014

High prevalence of hepatitis delta virus among patients with chronic hepatitis B virus infection and HIV-1 in an intermediate hepatitis B virus endemic region.

Saravanan S, Madhavan V, Velu V, Murugavel KG, Waldrop G, Solomon SS, Balakrishnan P, Kumarasamy N, Smith DM, Mayer KH, Solomon S, Thyagarajan SP. J Int Assoc Provid AIDS Care. 2014 Jan-Feb;13(1):85-90. doi: 10.1177/2325957413488166. Epub 2013 May 30.

PMID: 23722085

Research

With a modest beginning in research in 1999, YRGCARE has since emerged a pioneer in research on HIV and related co-infections, meeting the highest standards of ethics and regulatory compliance, in partnership with global leaders. Running across its clinic, community and laboratory clusters, these research studies answer questions that are most important and relevant to India in the fields of epidemiology, laboratory, prevention, natural history and treatment outcomes.

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 036	2005	HPTN 052 - A Randomized Trial to Evaluate the Effectiveness of Antiretroviral Therapy plus HIV Primary Care versus HIV Primary Care Alone to Prevent the Sexual Transmission of HIV-1 In Serodiscordant Couples	Dr N Kumarasamy	HPTN	DAIDS/NIAID
YRG - 056	2006	Optimizing HIV Care in Less Developed Countries	Dr N Kumarasamy	NIH	NIAID / NIH
YRG - 057	2004	Context and correlates of health behaviors in South India	Dr N Kumarasamy	NIH	NIAAA/NIH
YRG - 058	2006	Correlates of Mucosal HIV Transmission in Indian Women.	Prof.Suniti Solomon and Dr.K.G.Murugavel	University of California, USA.	R03, NIH, USA

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 059	2009	Cellular immunity in HIV infected individuals in India.	Dr.K.G.Murugavel.	Emory University.	Emory CFAR-USA
YRG - 078	2005	Changes in HIV Viral load in patients undergoing treatment for filarial infection	Dr N Kumarasamy	Johns Hopkins School of Public Health, Baltimore, MD; National Institute for Research in Tuberculosis, ICMR, Chennai, India	NIAID / NIH
YRG - 082	2005	HAART associated body composition and metabolic changes in HIV infected patients in South India	Suneeta Saghayam	Tufts University, Boston, MA, USA; Brown University, Providence, Rhode Island, USA	GRIP/NIH
YRG - 084	2006	A5230 - A Pilot Study of Lopinavir/Ritonavir in participants experiencing virologic relapse on NNRTI-containing regimens	Dr N Kumarasamy		NIAID / NIH
YRG - 087	2008	Ancillary Studies to Contribute to the Overall Goals of the Global HIV/AIDS Vaccine Enterpris (GHAVE) Comprehensive Antibody Vaccine Immune Monitoring Consortium (GCAV) for AIDS Vaccine Discovery Program: Phase 1.	Dr.P.Balakrishnan.	Duke University, USA.	Duke University – USA (Bill Melinda Gates Foundation).

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 090	2007	AMC Protocol # 054: A single-arm, open-label pilot study of the safety and immunogenicity of the Merck quadrivalent human papillomavirus vaccine among HIV-positive women in India. A Trial of the AIDS Malignancy: Clinical Trials Consortium (AMC)	Dr N Kumarasamy	AIDS Malignancy Consortium	National Cancer Institute (NCI), NIH, USA
YRG - 091	2007	A Multi-Center, Open label, Expanded Access Trial of Maraviroc (Protocol # A4001050)	Dr N Kumarasamy	Pfizer	Pfizer
YRG - 099	2009	Implementation of MODS for TB and MDRTB control – towards sustainable roll-out. MODS Study.	Prof.Suniti Solomon and Dr.P.Balakrishnan.	University of Peru.	World Bank.
YRG - 102	2007	Genetic Basis for HIV-1 Transmission, Resistance and Disease Progression: Polymorphism in MHC, Chemokine / Cytokine and their Receptor Genes	PI: Prof NK Mehra, AIIMS, New Delhi. Co-Investigator: Dr. Suniti Solomon	AIMS, New Delhi	ICMR-DBT
YRG - 106	2007	Targeting Mass Media Intervention for HIV, STD and Pregnancy Prevention in India	Dr Suniti Solomon	PSI	ICMR
YRG - 108	2007	'A Cross Sectional study to screen for and generate broadly neutralizing monoclonal antibodies from HIV infected individuals' - IAVI Protocol G	Dr. Suniti Solomon (PI - Study Site)	IAVI	IAVI, New Delhi

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 111	2009	HIV drug resistance in blood and genital tract in India. (Genital Study)	Dr.S. Saravanan.	Nil	Indian Council for Medical Research, New Delhi.
YRG - 120	2007	HIV & TB prevalence study- Nellore YRG Clinic	Dr Sunil Suhas Solomon		Intramural
YRG - 126	2008	BD Protocol: Collection of Clinical Sample Sets; Sputum, Blood and Urine, from Patients potentially infected with Mycobacterium Tuberculosis (Protocol number: BDT-BIO-07-0003)	Dr. Suniti Solomon	AGADA	
YRG - 129	2008	Comprehension of Informed Consent in Therapeutic Clinical Trials at YRGCARE	Dr. S. Swarnalakshmi	Nil	Intramural
YRG - 133	2008	The Treat Asia HIV Observational Database	Dr. N. Kumarasamy	TAHOD	TREATASIA
YRG - 134	2008	'The Treat Asia Pediatric HIV Observational Database (TApHOD)	Dr. N. Kumarasamy	TAPHOD, AMFAR	TREATASIA
YRG - 138	2008	Examining psychosocial and economic factors influencing ART Adherence in a cohort of HIV-infected individuals in South India	Mr. Vivek Murthy (PI), CUNY Medical School / Sophie Davis School of Biomedical Education, 160 Convent Ave. Harris Hall-408 New York, NY 10031		

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 139	2008	Malaria and HIV Co-infection in Southern India	Dr. Scott Letendre(PI), Dr. N. Kumarasamy & Dr Ajay R. Bharti (Co-investigators)		REACH Initiative - TIBOTEC
YRG - 140	2006	Development and Validation of a Pooled Serum PCR Assay for the Detection of Malaria among HIV Positive Individuals in Southern India	Dr David Smith (US PI) & Dr N. Kumarasamy (India PI)	UCSD, USA.	CFAR
YRG - 143	2008	Diversity of HIV and HCV Among IDUs in India (Administrative Supplement to DA12568 "Incidence of HIV in a Cohort of IV Drug Users" [Alive II])	Sunil S Solomon (PI), YRGCARE, Chennai, India; Shruti H. Mehta, Ph.D., MPH (PI), Associate Professor of Epidemiology, JHU, USA	JHU,USA	NIDA / NIH
YRG - 144	2008	A Phase III, Randomized, Double-blind Trial of TMC278 25 mg Q.D. versus Efavirenz 600 mg Q.D. in combination with a background regimen containing 2 nucleoside/nucleotide reverse transcriptase inhibitors in antiretroviral-naïve HIV-i infected subjects (Protocol No. TMC 278-TIDP6-C215)and its substudyA Substudy of TMC278-TIDP6-C215 to evaluate the effects of long-term TMC278 25 mg Q.D. and Efavirenz therapy on body fat and bone mineral density Protocol No. TMC278-TIDP6-C215Subdexa)	Dr N Kumarasamy	Represented by Johnson & Johnson	Tibotec Pharmaceuticals

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 148	2009	HIV, HCV and Liver Disease among Injection Drug users in Chennai, India	Dr. Suniti Solomon, Study PI, YRGCARE; Shruti H Mehta, Study PI, JHSPH	JHSPH, U.S.A	PA-07-275, Office of AIDS Research, NIH "International Research Collaboration on Drug Addiction"
YRG - 152	2009	Ariel Study – 'A Phase II Open Label Study to Evaluate Pharmacokinetics, Safety, Tolerability and Antiviral activity of DRV in Combination with Low-dose Ritonavir (DRV/rtv) in Treatment-experienced HIV-1 Infected Children from 3 years to < 6 years of age' (TMC 114-TiDP29-C228)	Dr N Kumarasamy	Represented by Johnson & Johnson	Tibotec
YRG - 154	2009	Adult PBMC Cryopreservation Proficiency Testing – Participation in Quality Assurance Programme	R. Vignesh		ACTG
YRG - 155	2009	"The antiretroviral roll out for HIV in India – Strengthening capacity to promote adherence and patient follow-up in the context - HIVIND.A research plan for a two-centre, randomized, controlled, open trial in South India to study an innovative approach to promote adherence in HIV+ Indian patients"	Dr N Kumarasamy		European Union (EU)

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 156	2009	Drug Resistance in different HIV-1 Subtypes (Sample Tanker Study)	Dr. S. Saravanan	Brown Univ, RI, USA Stanford Univ, SF, USA	National Institutes of Health, USA
YRG - 158	2009	'Anemia and Nutrition among Children with Perinatally Acquired HIV Infection in South India' (Childhood HIV & Anemia)	PI: Suneeta Saghayam, Co-PI: Dr N Kumarasamy		ICMR
YRG - 159	2009	Contingency Management in the delivery of HAART to Drug Users in Chennai, India ; Short Title: HIV DU Incentive Study	Sunil S Solomon (Study PI) ; Gregory M Lucas, Foreign PI, Johns Hopkins University School of Medicine, Baltimore, USA	Johns Hopkins University School of Medicine, Baltimore, USA	NIH (Supplement to DAART Study)
YRG - 160	2009	Preventing HIV among MSM in South India	Dr. Suniti Solomon (PI), Director, YRGCARE ; David D Celentano (PI), SCD, MHS (JHSPH)	Johns Hopkins University School of Medicine, Baltimore, USA	NIH
YRG - 164	2009	Serological Survey of Dengue and Chikungunya Infections among Individuals in Chennai, Tamil Nadu	Dr. Sunil Suhas Solomon (Study PI, YRGCARE) ; Dr. Derek Cummings, JHBSPH, Dr. Shruti Mehta, JHBSPH (Foreign PI)	Johns Hopkins University School of Medicine, Baltimore, USA	Burroughs Wellcome Fund (USA) Burroughs Wellcome Fund Career Award at the Scientific Interface (to Dr. Derek Cummings)

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 165	2009	Promoting HIV Prevention by understanding Fertility desires among HIV-positive women and men in India(Short Title: Fertility intentions and HIV in India)	Dr. Suniti Solomon	ICMR, St. John's Research Institute; Bangalore; Division of Reproductive Health and Nutrition, ICMR; Obstetrics & Gynecology, JJ Group of Hospitals, Mumbai	ICMR
YRG - 166	2009	Access and Acceptance of IT Solutions as Health Intervention Tools for Persons with HIV Disease in India, A Design Study - Substudy of Natural History Study	Dr. N. Kumarasamy, Study PI, YRGCARE; Other PI's: Prof. A. Joshi, Industrial Design Centre, IIT, Mumbai; Joris Van Dam, PhD, Johnson & Johnson Pharmaceutical Research & Development	European Union	Johnson & Johnson, India
YRG - 167	2009	A Survey of Sexual Risk Taking Practices, Substance Abuse and VCT Seeking Practices among Adults of the Middle- and High-Income Groups ; Short Title: Middle and High-Income Group Survey	Dr.Sunil Suhas Solomon, Study PI, YRGCARE; Aylur K Srikrishnan (YRGCARE)		Intramural

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 168	2009	A Randomised Open-Label Study comparing the Safety and Efficacy of Ritonavir boosted Lopinavir and 2-3N(T)RTI Backbone versus Ritonavir boosted Lopinavir and Raltegravir in Participants Virologically Failing First-Line NNRTI/2N(T)RTI Therapy: The Second-Line Study	Dr. N. Kumarasamy		National Centre in HIV Epidemiology and Clinical Research (NCHECR), University of New South Wales, (UNSW), Merck Sharp & Donhme and Abbott
YRG - 169	2009	Prevalence of and Factors Affecting Tobacco use and readiness to quit amongst HIV positive individuals in Tamil Nadu India,	Timothy Poore	Brown University, Providence, RI, USA	Clinical Research Scholars Program, NIH, US;Fogarty Scholar, Brown University, Providence, Rhode Island, USA
YRG - 173	2009	A PHASE IV, Observer-Blind, Randomized, Controlled, Multicentric Study to Assess the Safety and Immunogenicity of GSK Biologicals' HPV -16/18 L1 VLP AS04 Vaccine (Cervarix) Administered Intramuscularly According to A 3-Dose Schedule (Month 0, Week 6, Month 6) in Human Immunodeficiency Virus-Infected (HIV +) Female Subjects Aged 15-25 Years, as Compared to Merck's HVP 6/11/16/18 Vaccine (Gardasil) (HPV-019 PRI)	Dr. N. Kumarasamy	GlaxoSmithKline Biologicals, Belgium	GlaxoSmithKline Biologicals

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 174	2009	Evaluation of Host Immune Responses to the Tat Antigen of HIV-1 in the Indian Clinical Cohorts	Dr. Uday Kumar Ranga, (JNCASR), Bangalore (PI) Dr Suniti Solomon (Co PI)	DBT, GOI	JNCASR, Bangalore
YRG - 175	2009	Age-Related Blood Lymphocyte Subsets in Healthy Indian Children – A Multicentric Study	Dr. P. Balakrishnan (Study PI) ; Dr Suniti Solomon (Co PI)	DBT, GOI	NACO
YRG - 176	2010	Development of Anti-Viral Micro RNA Based Prognostic Biomarker for HIV-1 Infection- A Pilot Study	Dr Suniti Solomon (Study PI); Dr. M Sundaram (Co-PI) Dr P Balakrishnan	CSIR-Institute of Genomics and Integrative Biology, New Delhi	Dept of Biotechnology, Govt. of India
YRG - 179	2010	113935 (Tuberculosis-014 Clinical Trial)Title: Safety and Immunogenicity of GSK Biologicals' Candidate Tuberculosis Vaccine (692342) when administered to HIV-Positive Adults Living in a TB Endemic RegionDetailed Title: A Phase II, Double Blind (Observer-Blind), Randomised, Controlled Study to Evaluate the Safety, Reactogenicity and Immunogenicity of GSK Biologicals' Candidate Tuberculosis Vaccine (m72/AS01E) when Administered Intramuscularly according to a 0, 1 Month Schedule to HIV-Positive Adults Living in a TB Endemic Region	Dr. N. Kumarasamy (Study PI)	GlaxoSmithKline Biologicals, Belgium	GlaxoSmithKline Biologicals

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 180	2010	Examination of the Use of Traditional Medicines for HIV/AIDS In India	Dr. J Sastry (Study PI), B J Medical College, Pune	B J Medical College, Pune	
YRG - 181	2010	The Encore1 Study : A Randomised, Double-Blind, Placebo-Controlled, Clinical Trial to Compare the Safety and Efficacy of Reduced Dose Efavirenz (EFV) with Standard Dose EFV Plus Two Nucleotide Reverse Transcriptase Inhibitors (N(T)Rti) in Antiretroviral-Naïve HIV-Infected Individuals Over 96 Weeks The Encore Program: Evaluation of Novel Concepts in Optimization of Antiretroviral Efficacy	Dr. N. Kumarasamy (Study PI)	Univ of New South Wales, Australia	NCHECR, University of New South Wales, Australia
YRG - 182	2010	Hepatitis B Virus Infection in a Global Cohort of HIV-1 infected subjects and their partners	Alexandra Valsamakis, M.D., Ph.D Shanmugam Saravanan, Ph.D. N. Kumarasamy, MBBS, PhD. Suniti Solomon, MD	Johns Hopkins University, Baltimore, USA	HPTN / NIH

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 183	2010	Understanding issues around disclosure, intervention processes and expectations of HIV prevention approaches targeted at Men having Sex with Men	Mr. Aylur Kailasam Srikrishnan (PI); Dr Sunil Suhas Solomon (PI)		
YRG - 184	2010	Ganciclovir-Intravitreal Versus Systemic-Cytomegalovirus Retinitis Trial (GIVCYT)	Dr Jyotirmay Biswas (Study PI), Dr. N. Kumarasamy (Co-Investigator),, Dr S Sudharshan; Dr John Kempen	Sankara Nethralaya, Chennai	External - SankaraNethralaya
YRG - 185	2010	A Phase III, Open-Label, Randomized, Assessment-Blinded Clinical Trial to Compare the Safety and Efficacy of Topical Gentian Violet to that of Nystatin Oral Suspension for the Treatment of Oropharyngeal Candidiasis in HIV-1 Infected Participants in Non-U.S. Settings Short Title for the Study: Compare the Efficacy and Safety of Gentian Violet to that of Nystatin	Principal Investigator: Dr. K. Ranganathan, M.D.S., M.S (Ohio) PhDInvestigator: Dr. N. Kumarasamy	ACTG Network	NIAID / NIDCR / NIH

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 186	2010	The detection of Occult Hepatitis B Virus infection using Real-time PCR on Pooled Serum Samples among HIV positive individuals in Southern India	Dr. S. Saravanan., Ph.D., Dr. Vidya M., Ph.D., Prof. Suniti Solomon	UCSD CFAR	Indo-US Bilateral Project ICMR, New Delhi & National Institutes of Health (NIH), USA
YRG-192	2010	Married Women in Tamil Nadu India with HIV: Barriers to Anti-Retroviral Adherence	Krutika Kuppalli M.D	Emory University School of Medicine Division of Infectious Disease Atlanta, USA	Fogarty International Clinical Research Fellows Program
YRG-193	2010	A5271022: "A Phase 2B multicenter, randomized, comparative trial of UK-453,061 versus Etravirine in combination with Darunavir/Ritonavir and a nucleotide/nucleoside reverse transcriptase inhibitor for the treatment of antiretroviral experienced HIV-1 infected subjects with evidence of NNRTI resistant HIV-1" Protocol Number: A5271022 Phase: 2B	Dr N Kumarasamy	Pfizer	Pfizer

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG-196	2010	Continued Access to darunavir/ritonavir (DRV/rtv) in HIV-1 infected children and adolescents aged 3 years and above (Trial No. TMC114-TiDP29-C232)	Dr N Kumarasamy	AIIMS, New Delhi	Dept of Biotechnology, ICMR Pfizer
YRG-198	2010	Psychological Correlates and Management of Menstrual Distress among HIV Infected Patients	Dr. Poongulali Selvamuthu, Research Scholar for PhD	Project for PhD thesis	Self
YRG-200	YYYY	Menstrual Irregularities in Indian Women with HIV: Is Immunologic Status a Factor?	Krutika Kuppalli M.D Poongulali Selvamuthu, , MBBS, Msc	Emory University School of Medicine, Division of Infectious Disease, 49 Jesse Hill JR Drive, Atlanta, USA	
YRG-201	2010	A0081251 entitled – “Phase 3B - An open-label, extension safety trial of Pregabalin in subjects with neuropathic pain associated with HIV neuropathy (Pregabalin A0081251)” Protocol Number: A0081251 Phase: 3B	Dr N Kumarasamy	Pfizer	Pfizer Ltd. Mumbai

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG-202	2010	Neurocognitive and behavioral effects of HIV/AIDS on children attending a non-government hospital in South India	Samara Rifkin (PI and Study Coordinator)	Boston University School of Medicine	Fogarty International Clinical Research Programs
YRG-206	2011	Bone and Body Composition - A Sub Study of the Second-Line Study- A Randomised Open-Label Study Comparing the Safety and Efficacy of Ritonavir Boosted Lopinavir and 2-3N(t)RTI Backbone Versus Ritonavir-Boosted Lopinavir and Raltegravir in participants virologically failing First-Line NNRTI/2N(t)RTI Therapy: The Second-Line Study	Dr. N. Kumarasamy		National Centre in HIV Epidemiology and Clinical Research (NCHECR) University of New SouthWales (UNSW)Sydney, Australia
YRG-210	2011	Integrated Care Clinics for IDUs in India: A Cluster Randomized Trial	Aylur K. Srikrishnan	JHU	NIDA, NIH
YRG-213	2010	Immunological correlates of Lack of Disease progression in HIV-1 subtype C Infected Long Term Non-Progressors from India.	Dr.K.G. Murugavel	Nil	Indian Council for Medical Research, New Delhi.
YRG-214	2011	Changes in Mitochondrial DNA as a marker of Nucleoside Toxicity Among HIV-Infected Individuals in Southern India LTNP Study	Dr P Balakrishnan		ICMR

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG-215	2011	Collection of Comparison Neurocognitive Data in Resource-Limited Settings: A Limited-Center Trial of the AIDS Clinical Trials Group (A5271)	Dr N Kumarasamy	ACTG Network Trial	DAIDS/NIAID
YRG-216	2011	A5273: Multicenter Study of Options for Second-Line Effective Combination Therapy (SELECT), A Multicenter Trial of the AIDS Clinical Trials Group	Dr N Kumarasamy	ACTG Network Trial	DAIDS/NIAID
YRG-217	2011	A5274/REMEMBER: Reducing Early Mortality and Early Morbidity by Empiric Tuberculosis Treatment Regimens: A Site-limited Trial of the AIDS Clinical Trials Group (A5274)	Dr. N. Kumarasamy	ACTG Network Trial	DAIDS/NIAID
YRG-218	2011	Losses-to-Follow up in a Tertiary HIV CARE Facility in Chennai, India, Protocol	Dr Suniti Solomon		Intramural
YRG-219	2011	Immune Profiling of IRIS in HIV/TB co-infection	Dr KG Murugavel		ICMR, New Delhi
YRG-224	2011	Phase III Clinical Trial of Ultra-Short-Course Rifapentine/Isoniazid for the Prevention of Active Tuberculosis in HIV-Infected Individuals with Latent Tuberculosis Infection, (Protocol No. A5279)	Dr N Kumarasamy	ACTG Network Trial	ACTG/NIH

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG-225	2011	Strategic Timing of Antiretroviral Treatment (Insight Protocol 001: START)	Dr N Kumarasamy	INSIGHT Network	NIH, USA; France; Germany; Australia
YRG-226	2011	Exploring Physical, Reproductive and Mental Health Care needs of Female Sex Workers in Chennai, India	AK Srikrishnan		Intramural
YRG-229	2011	A5288: Management Using the Latest Technologies in Resource-limited Settings to Optimize Combination Therapy After Viral Failure (MULTI-OCTAVE) - A Multicenter International Trial Of The Aids Clinical Trials Group (ACTG)Short Title for the Study: Multi-Octave (Management using latest Technologies to Optimize Combination Therapy after Viral Failure) (Non-IND)	Dr N Kumarasamy	ACTG Network Trial	DAIDS/NIAID
YRG-230	2011	A Randomised, Double Blind, Placebo Controlled, Clinical Trial to compare the Safety and Efficacy of Reduced Dose Efavirenz (EFV) with Standard Dose EFV Plus 2N(T)RTI in Antiretroviral-Naïve HIV-Infected Individuals over 96 Weeks: the Neurocognitive Sub-Study (Encore Substudy)	Dr N Kumarasamy	Univ of New South Wales, Australia	NCHECR, University of New South Wales, Australia

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 233	2011	Arterial Elasticity: A Substudy of Strategic Timing of AntiRetroviral Treatment	Dr N Kumarasamy	INSIGHT Network	NIAID, NIH,USA; France;Germany;
		(START): A Multicenter Study of the International Network for Strategic Initiatives in Global HIV Trials (INSIGHT PROTOCOL 001D: START Arterial Elasticity Substudy)			Australia
YRG-235	2011	A Randomised, Double-Blind, Multi-Centre, Parallel-Group Phase 3B Study to Demonstrate Non-Inferiority of Stavudine (20 Mg Twice Daily) compared with Tenofovir Disoproxil Fumarate (300 Mg Once Daily) when administered in combination with Lamivudine and Efavirenz in Anti Retroviral-Naive Patients Infected with HIV-1 (Protocol No. WRHI 001)	Dr N Kumarasamy		Bill & Melinda Gates Foundation
YRG-236	2012	HIV/AIDS in India: Individual, Family, Community, Caste group	Caitlin O'Grady (PI), Researcher, University College London		Self

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG-238	2012	A Pilot Study on social life, stigma, and societal engagement among a cross section of transgenders in Chennai, India	Aylur Kailasam Srikrishnan (Site Investigator), Research Manager, YRG CARECC: Professor Richard Tewksbury, Ph.D., Department of Justice Administration, University of Louisville Assistant Professor Geetha Suresh, Ph.D., Department of Justice Administration, University of Louisville	Department of Justice Administration, University of Louisville, US	
YRG-240	2012	Effectiveness of HAART in a public-private HIV clinic in South India	Shruti Mehta, MPH, PhD (Study PI), JHBSPH; Ms. Greer Waldrop (Study Coordinator), JHBSPH	JHBSPH, US; Asha Kirana, Mysore	

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG-241	2012	A5297: An Open-Label, Proof of Concept, Randomized Trial comparing a LPV/R-Based to an NNRTI-Based Antiretroviral Therapy Regimen for Clearance of Plasmodium Falciparum Subclinical Parasitemia in HIV-infected Adults with CD4+ Counts >200 and <350 cells/mm ³ : A Limited Center Trial of the AIDS Clinical Trials Group (A5297) (NON-IND PROTOCOL)	Dr. Nagalingeswaran Kumarasamy, Study PI, YRGCARE	ACTG Network Trial	ACTG
YRG-245	2012	HIV Drug Resistance Monitoring in Chennai, India	Dr. S. Saravanan (Study PI - India), YRGCARE ; Dr. Katzenstein, David A (Study PI - US), Stanford University Medical Center, Stanford California 94305 ; Dr. Rami Kantor (Study PI - US) Brown University Alpert Medical School The Miriam Hospital, Providence, RI 02906	Dr Rami Kantor ,Brown University, Rodhe Island , USA and Dr Katzenstein, Stanford University,USA	Indo-US R21 ICMR, New Delhi & NIH, USA

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG-246	2012	Evaluation of AlereTM DetermineTM TB LAM AG Rapid Test for Diagnosis of TB Infection	Dr. R. Vignesh (Study PI), Co-Investigators (YRGCARE, Ch.): Dr. P. Balakrishnan, Prof. Suniti Solomon, Dr. Sunil Suhas Solomon; Co-Investigators (Asha Kirana Hospital, Mysore): Dr. S.N. Mothi, Dr. V.H.T. Swamy, Dr. Srirama B. Rao	Asha Kirana, Mysore	Alere
YRG-248	2012	Evaluation of Point of Care CD4 Assay (VISITECT®) for monitoring HIV Patients in Resource-Limited Settings	Dr. P. Balakrishnan (Study PI), YRGCARE	Burnet Institute, Melbourne, Australia	Omega Diagnostics, Mumbai
YRG-249	2012	Validation Study of ReaSLR Sputum Concentration Method for AFB Microscopy	Dr. R. Vignesh (Study PI)		Reagenix, Bangalore
YRG-250	2012	Pulmonary Substudy: A Substudy of Strategic Timing of AntiRetroviral Treatment (START): A Multicenter Study of the International Network for Strategic Initiatives in Global HIV Trials (INSIGHT Protocol 001E: START Pulmonary Substudy)			

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG-253	2013	A5290 - A Randomized, Phase 2B Study of A Double-Dose Lopinavir/Ritonavir-based Antiretroviral Regimen with Rifampin-based Tuberculosis Treatment versus a Standard-Dose Lopinavir/Ritonavir-based Antiretroviral Regimen with Rifabutin-based Tuberculosis Treatment with or without Raltegravir in HIV-1-Infected persons requiring treatment for active TB and HIV - A Multicenter Trial of the AIDS Clinical Trials Group (ACTG)	Dr. N. Kumarasamy, Study PI	ACTG Network trial	DAIDS /NIAID
YRG-254	2013	Camp Rainbow and follow up Program outcomes evaluation	Dr. Suniti Solomon, Study PI	SeriousFun	SeriousFun Children's Network, USA
YRG-255	2013	Assessing Barriers and Facilitators to Pediatric HIV Status Disclosure from the perspective of Healthcare Providers in South India	Rajitha Devadoss, MD (Study PI), Yale University (New Haven, CT, USA)	Yale University	Self
YRG-256	2013	South India Domestic Violence Study: Prevalence of Domestic Violence and its Correlates and Determinants with HIV Infection among South Indian Women	Ms. Dhanalakshmi Thirumalai, PhD Candidate, UCLA School of Public Health 3300 Sawtelle BLVD, APT 104 Los Angeles, CA-90066	External Student	Self

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG-257	2013	Validation of Rapid Point of Care CD4 Testing (VISITECT®) in Resource-Limited Settings	Dr. P. Balakrishnan, PhD (Study PI), Laboratory Manager, (Chief / Quality), YRGCARE Infectious Diseases Laboratory	Burnet Institute, Melbourne, Australia	Omega Diagnostics, Mumbai
YRG-258	2013	Acceptability of mobile phone reminders for improving adherence to antiretroviral therapy: Participant experiences within a randomised controlled trial (Short Title: EU HIVIND Sub-Study)	Dr Rashmi Rodrigues and Dr.N.Kumarasamy	St. John's Medical College, Bangalore	European Union
YRG-260	2013	Identifying Mirna Clusters In Retinal Cytokine Producing Glia Which Control Inflammation During HIV1 Tat Exposure: Implications For Immune Recovery Uveitis	Dr.Nivedita Chatterjee	Vision Research Foundation, Sankara Nethralaya	Department of Biotechnology, Dr.Dhananjay Tiwary , Block 2, 6 th Floor, CGO Complex, Lodhi Road. New Delhi
YRG-261	2014	An Evaluation of Auto CD4/CD4% Assay using EMD Millipore Muse® Cell Analyzer	Dr P Balakrishnan	EMD Millipore Muse® Cell Analyzer	

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 263	2014	A Phase 3, Randomized, Double-Blind Study to Evaluate the Safety and Efficacy of Tenofovir Alafenamide (TAF) 25 mg QD versus Tenofovir Disoproxil Fumarate (TDF) 300 mg QD for the Treatment of HBeAg-negative, Chronic Hepatitis B, Protocol ID/Number: GS-US-320-0108	Dr. Suniti Solomon Dr. Pradeep Amrose		Gilead
YRG - 264	2014	A Phase 3, Randomized, Double-Blind Study to Evaluate the Safety and Efficacy of Tenofovir Alafenamide (TAF) 25 mg QD versus Tenofovir Disoproxil Fumarate (TDF) 300 mg QD for the Treatment of HBeAg-Positive, Chronic Hepatitis B, Protocol ID/Number: GS-US-320-0110	Dr. Suniti Solomon Dr. Pradeep Amrose	KlinEra Corporation India	Gilead / KlinEra Corporation India
YRG - 265	2014	Impact of Depression and Syndemic Psychosocial Health conditions on Patients Living with HIV/AIDS in Southern India	Brian T. Chan, MD Kenneth Mayer, MD N. Kumarasamy, M. Suresh Kumar, Lakshmi Prasad	Brigham and Women's Hospital / Massachusetts General Hospital / Harvard University	Self
YRG - 268	2013	Project UNITE: A randomized, controlled trial to assess the impact of TAMA (Health IT System) on treatment effectiveness in HIV-1 infected subjects initiated on first-line antiretroviral therapyClinical Trial	Dr. N. Kumarasamy, Study PI, YRGCARE; Study Sub-Investigator: Dr. S. Poongulali, YRGCARE	JANSSEN INFECTIOUS DISEASES – DIAGNOSTICS BVBA	JANSSEN INFECTIOUS DISEASES – DIAGNOSTICS BVBA

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 269		Indian HIV Clinicians' Attitudes Toward Depression In Plhiv	Brian T. Chan, Md, Brigham And Women's Hospital / Massachusetts General Hospital / Harvard University		
YRG - 266	2012	Development of a National HIV Drug Resistance Database by Generating and Utilizing Clinical, Genotypic and Phenotypic Characterization Data of the Prevalent Drug Resistant Strains of Indian HIV-1 Clade C	Suniti Solomon, MD, Study PI, Director, YRGCARE CC: Study Coordinators: P.Balakrishnan, Ph.D., Associate Professor, YRGCARE. Sunil Suhas Solomon, MBBS, MPH, Ph.D., Senior Medical Officer, YRGCARE M.P.Subha, M.Sc, Research Assistant, YRGCARE	NARI, Pune	ICMR

PROJECT NO.	YEAR	TITLE	PI	COLLABORATION	FUNDING
YRG - 267	2012	Human Immunodeficiency Virus Type-1(HIV-1) Pol Gene Diversity among Patients on Antiretroviral Therapy	S. Sivamalar, M.Sc., Study PI, Research Assistant, YRGCARE Co- Investigators: S Saravanan, PhD., Assistant Professor, YRGCARE P. Balakrishnan, PhD., Associate Professor, YRGCARE Prof. Suniti Solomon, MD., Director, YRGCARE	PhD Thesis Madras University	Intramural
YRG - 268	2013	Project UNITE: A randomized, controlled trial to assess the impact of TAMA (Health IT System) on treatment effectiveness in HIV-1 infected subjects initiated on first-line antiretroviral therapyClinical Trial	Dr. N. Kumarasamy, Study PI, YRG CARE; Study Sub-Investigator: Dr. S. Poongulali, YRG CARE	JANSSEN INFECTIOUS DISEASES – DIAGNOSTICS BVBA	Janssen Infectious Diseases – Diagnostics BVBA
YRG - 269	2014	Indian HIV Clinicians' Attitudes Toward Depression In Plhiv	Brian T. Chan, Md, Brigham And Women's Hospital / Massachusetts General Hospital / Harvard University	Brigham And Women's Hospital / Massachusetts General Hospital / Harvard University	Self

Summary of Financial Statements

Consolidated Balance Sheet (2007-14)

Financial Highlights	Rupees in Lacs							
Balance Sheet as at 31 March	2007	2008	2009	2010	2011	2012	2013	2014
Sources of Fund								
General Fund	1169.67	1405.52	1790.08	2319.88	2541.81	2708.98	2998.94	3526.20
Grants in Advance	434.07	558.65	567.45	441.71	369.19	450.76	589.93	811.31
Capital Reserve Fund	186.77	200.34	214.17	99.15	189.17	222.33	269.03	273.36
Endowment Funds	345.21	365.67	335.91	321.12	338.91	307.90	351.47	362.33
Current Liabilities & Provisions	30.80	8.46	28.85	12.31	39.70	26.92	36.99	21.73
Total	2166.52	2538.64	2936.46	3194.17	3478.78	3716.89	4246.36	4994.93

Application of Fund								
Fixed Assets - Designated	186.77	200.34	214.17	99.15	189.17	222.33	254.93	259.26
Fixed Assets - Non Designated	293.85	289.68	468.25	795.35	810.59	811.63	777.88	737.57
Investments - Endowment Fund	344.8	364.11	336.85	317.53	343.25	302.25	347.31	361.33
Investments - Others	910.30	1280.09	1525.20	1341.44	1421.17	1473.40	1788.94	2477.80
Grants in Arrears	361.09	337.83	264.83	530.54	590.07	472.81	623.78	197.14
Advance & Deposits, Receivables	29.53	37.61	79.75	65.01	87.71	111.01	116.44	134.01
Cash & Bank	40.18	28.98	47.41	45.15	36.82	323.46	337.08	827.82
Total	2166.52	2538.64	2936.46	3194.17	3478.78	3716.89	4246.36	4994.93

Consolidated Income and Expenditure Account (2007-14)

Income & Expenditure A/C	Rupees in Lacs (Year ended 31 March)							
Income	2007	2008	2009	2010	2011	2012	2013	2014
Grant Income (foreign contributions)	762.05	1066.92	1315.55	1478.34	1470.32	1134.86	2061.21	2335.16
Grant Income (within India)	67.35	119.96	122.14	102.42	123.75	149.61	119.97	66.65
Donation, Reimbursements & Others	97.69	115.78	221.96	338.02	249.84	240.88	374.17	396.52
Net income from operations	129.38	148.40	224.32	328.86	344.54	459.20	403.31	331.16
Total (A)	1056.47	1451.06	1883.97	2247.64	2188.45	1984.55	2958.66	3129.49

Expenditure								
FCRA Grants - project expenses	762.05	1066.92	1315.55	1478.34	1470.32	1134.86	2061.21	2335.16
India Grants - project expenses	67.35	119.96	122.14	102.42	123.75	149.61	119.97	66.65
Administrative and other expenses	23.21	16.61	134.32	265.62	296.54	494.89	374.34	111.09
Depreciation	15.59	17.59	16.36	33.88	75.90	73.72	73.18	66.80
Total (B)	868.20	1221.08	1588.37	1880.26	1966.51	1853.08	2628.70	2529.70
Excess of income over expenditure	188.27	229.98	295.60	367.38	221.94	131.47	329.96	549.79

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